Supreme Court, U.S.
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No. _____ OFFICE OF THE CLERK

IN THE

Supreme Court of the United States

OCTOBER TERM, 1996

HUMANA INC., and
HUMANA HEALTH INSURANCE OF NEVADA, INC.,
Petitioners,

v.

MARY FORSYTH, MARIETTA CADE,
WILLIE ANDREWS, MARY LOU BUEHLER,
HELEN STAVES, RANDOLPH BRATTON,
AND SEARLE AUTO GLASS, INC.,
Respondents.

On Writ of Certiorari to the United States Court of Appeals for the Ninth Circuit

PETITION FOR WRIT OF CERTIORARI

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QUESTION PRESENTED

Whether the application of a federal statute -- here the Racketeer Influenced and Corrupt Organizations Act ("RICO") -- to the business of insurance would "invalidate, impair or supersede" state law regulating the same conduct in contravention of the McCarran-Ferguson Act where the state and federal statutory prohibitions are parallel, but the remedies provided are materially different.

PARTIES TO THE PROCEEDING BELOW

1. Humana Inc. ("Humana") and Humana Health Insurance of Nevada, Inc. ("Humana Insurance"), petitioners herein, were defendants in the district court and appellees in the court of appeals. Humana Insurance is a wholly owned subsidiary of Humana. There are no other parent corporations of petitioners and no nonwholly owned subsidiaries; but, by reason of certain contractual obligations undertaken in connection with the acquisition of Humana's former hospital operations subsequent to the events at issue herein, Columbia/HCA Healthcare Corporation is beneficially interested in the outcome of this litigation.

2. Mary Forsyth, Marietta Cade, Willie Andrews, Mary Lou Buehler, Helen Staves, Randolph Bratton and Searle Auto Glass, Inc., respondents here, were plaintiffs in the district court and appellants in the court of appeals. They were certified by the district court pursuant to Rule 23 of the Federal Rules of Civil Procedure as representatives of the classes described herein.

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In The

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October Term, 1996

Humana Inc., and
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Mary Forsyth, Marietta Cade,
Willie Andrews, Mary Lou Buehler,
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On Writ of Certiorari to the United States Court of Appeals for the Ninth Circuit

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PETITION FOR WRIT OF CERTIORARI

REPORT OF OPINIONS BELOW

The opinion of the court of appeals is reported as *Forsyth v. Humana, Inc.*, 114 F.3d 1467 (9th Cir. 1997). That opinion withdraws and supersedes an earlier opinion of the same court reported as *Forsyth v. Humana, Inc.*, 99 F.3d 1504 (9th Cir. 1996). The opinion of the district court is reported as *Forsyth v. Humana, Inc.*, 827 F. Supp. 1498 (D. Nev. 1993). The decisions of the court of appeals and the

district court are found as Appendices A and B hereto respectively.

JURISDICTION

The decision for which review is sought was entered by the court of appeals on May 23, 1997. That decision granted in part a timely motion for rehearing filed by petitioners here respecting an earlier decision entered by the court of appeals on November 5, 1996. This Court has jurisdiction to review the decision at issue by writ of certiorari pursuant to 28 U.S.C. §§ 1254(1) and 2101(c).

STATUTES AT ISSUE

This petition addresses principally the interpretation of section 2(b) of the McCarran-Ferguson Act, 15 U.S.C. § 1012(b), which provides, in pertinent part:

> "No Act of Congress shall be construed to invalidate, impair or supersede any law enacted by any State for the purpose of regulating the business of insurance . . . unless such Act specifically relates to the business of insurance."

The civil remedy provisions of RICO, 18 U.S.C. §§ 1961-1968 are also implicated by this petition. The full texts of all pertinent statutory provisions are set forth in Appendix C hereto.

STATEMENT OF THE CASE

A. Jurisdiction Below.

The district court had jurisdiction pursuant to 28 U.S.C. § 1331.

B. Pertinent Facts and Procedural History.

This action arises out of disputes respecting the administration and interpretation of employee group health insurance policies sold by Humana Insurance in the mid-1980's in the State of Nevada. These policies contained a "co-payment" provision pursuant to which patients covered by the policies were obligated to pay all charges up to a specified deductible and to pay 20% of charges above the deductible (up to a specified annual maximum). Humana Insurance was responsible for the remaining 80%.

Among other coverages, the policies covered treatment at certain Nevada hospitals, including a hospital owned by Humana, Humana Hospital-Sunrise ("Sunrise"). Humana Insurance negotiated discounts with Sunrise for hospital services provided to its policy beneficiaries, substantially reducing the costs on many procedures. In administering the co-payment feature of the policies, Humana Insurance applied those discounts to its 80% obligation. Policy beneficiaries continued to be billed for their 20% of the undiscounted hospital charge.

On this nucleus of facts, plaintiffs have asserted three claims for relief: (1) a claim under the Employee Retirement Income Security Act of 1974 ("ERISA"), 29 U.S.C. § 1001 *et seq.*; (2) a claim under the Sherman Act, 15 U.S.C. § 1 *et seq.*; and (3) a RICO claim. These claims are urged on behalf of two classes certified by the district court -- a class of the employers who bought the policies (the "Premium Payor Class") and a class of the employees who made co-payments under the policies (the "Co-Payor Class").

The opinions below address all three claims,[1] but only the RICO claim is at issue here.

The RICO claim is predicated upon the allegation (denied by petitioners and not yet tried below) that Humana Insurance concealed or misrepresented its discount arrangements with Sunrise in various mailings, telephone calls and television or radio advertisements directed at both classes. The purpose of these alleged acts of mail and wire fraud, according to plaintiffs, was to defraud the employer members of the Premium Payor Class into purchasing the policies and to defraud the employee members of the Co-Payor Class into making excessive co-payments. Humana and Humana Insurance moved the district court for summary judgment on this RICO claim on the grounds, *inter alia*, that the application of RICO to the insurance practices at issue was barred by section 2 of the McCarran-Ferguson Act.

The district court held that RICO was subject to the limitations of the McCarran-Ferguson Act, that the conduct alleged was part of the business of insurance and that the conduct was subject to Nevada's comprehensive regulatory

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[1] The district court granted summary judgment on all claims. The Ninth Circuit affirmed the district court's grant of summary judgment on the ERISA claim and reversed the grant of summary judgment on the Sherman Act claim. While petitioners believe that the latter ruling was erroneous, they do not urge that the issue as currently framed by the Ninth Circuit decision satisfies the criteria for review set forth in this Court's Rule 10.

scheme governing the business of insurance in that state which reserves to the Nevada Commissioner of Insurance "exclusive jurisdiction" in regulating the trade practices of the insurance industry. Appendix B at 91a-97a. (827 F. Supp. at 1520-21.) The district court also concluded that application of RICO to those practices would impair Nevada's regulatory scheme because RICO's civil penalties -- principally its treble damages provision -- far exceed the penalties proscribed by the State of Nevada for the same conduct: "For this Court to allow Plaintiffs to pursue the harsher RICO penalties for behavior which the Nevada Legislature apparently did not deem worthy of such penalties, would be tantamount to allowing Congress to intercede in an area expressly left to the states under the McCarran-Ferguson Act." Appendix B at 95a. (827 F.Supp. at 1521-22.) On this reasoning, the district court granted summary judgment on the RICO claim.[2]

On appeal, the Ninth Circuit reversed the district court on this issue. The court of appeals agreed with the district court's determination that the conduct at issue was the business of insurance, that Nevada regulated the conduct and that RICO was subject to the limitations of the McCarran-

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[2] The district court also granted summary judgment on the RICO claim as to the Premium Payor Class on the alternative ground that the members of that class could not show a causal nexus between the alleged predicate acts and the damages claimed. Appendix B at 86a-90a. (827 F.Supp. at 1518-20.) This ruling was affirmed by the Ninth Circuit. Appendix A at 26a-28a. (114 F.3d at 1481-82.)

Ferguson Act. Appendix A at 22a-25a. (114 F.3d at 1479-80.) The court of appeals did not agree, however, that the application of RICO would impair Nevada's regulatory scheme. Relying upon its recent decision in *Merchants Home Delivery Service, Inc. v. Frank B. Hall & Co.*, 50 F.3d 1486 (9th Cir.), *cert. denied* __ U.S. __, 116 S.Ct. 418 (1995), the court of appeals held that the disparity in remedies between RICO and Nevada insurance law did not create a conflict between federal and state law sufficient to preclude plaintiffs' RICO claim. Appendix A at 25a-26a. (114 F.3d at 1480.)

REASONS FOR GRANTING THE WRIT

Review by this Court is necessary to resolve a conflict between the circuits on the question presented by this petition. As noted, the Ninth Circuit has concluded that the McCarran-Ferguson Act does not preclude the application of federal law to the business of insurance where the federal and state statutes are parallel in their prohibitions, even though the consequences of violation of the federal statute are materially greater than those for violation of the state statute. The Sixth and Eighth Circuits have reached precisely the opposite conclusion, with both holding, as the district court did here, that application of the treble damages remedy allowed by RICO would conflict with the decision of the state legislatures to impose lesser penalties and, hence, impair the state regulatory scheme. *Doe v. Norwest Bank Minnesota*, 107 F.3d 1297, 1306-08 (8th Cir. 1997); *Kenty v. Bank One, Columbus N.A.*, 92 F.3d 384, 392-93 (6th Cir. 1996). In addition, although it has published no opinion explaining its reasoning, the Fourth Circuit has upheld a

district court ruling that the McCarran-Ferguson Act bars application of RICO to the business of insurance, affirming *per curiam* a decision to that effect in the Eastern District of Virginia. *Ambrose v. Blue Cross & Blue Shield*, 891 F. Supp. 1153, 1165-68 (E.D. Va. 1995), *aff'd mem.*, 95 F.3d 41 (4th Cir. 1996).[3]

The Ninth Circuit is alone among the Circuits in holding that RICO's remedies do not create a conflict with less draconian state laws. The Sixth and Seventh Circuits have, however, upheld the application of federal statutes whose remedies differ less significantly than do RICO's from the state laws that they parallel. *Nationwide Mut. Ins. Co. v. Cisneros*, 52 F.3d 1351, 1363 (6th Cir. 1995), *cert. denied*, __ U.S. __, 116 S.Ct. 973 (1996) (Fair Housing Act);[4] *NAACP v. American Family Mut. Ins. Co.*, 978 F.2d 287, 295-97 (7th Cir. 1992), *cert. denied*, 508 U.S. 907 (1993) (Fair Housing Act).

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[3] District court decisions to the same effect may also be found in the Third Circuit. *Wexco Inc. v. INC, Inc.*, 820 F. Supp. 194, 203-06 (N.D. Pa. 1993); *Senich v. Transamerica Premier Ins. Co.*, 766 F. Supp. 339, 341-42 (W.D. Pa. 1990).

[4] In *Kenty v. Bank One*, *supra*, 92 F.3d at 392, the Sixth Circuit distinguished its holding there that the McCarran-Ferguson Act bars the application of RICO from its earlier decision in *Cisneros* respecting the Fair Housing Act by noting that the differences between that statute and the parallel state law were principally procedural -- *i.e.*, the availability *vel non* of trial by jury -- rather than substantive.

The disparity among the treatments afforded to this issue by the circuits reflects the paucity of guidance to be found in the prior decisions of this Court. As the Eighth Circuit noted in *Doe v. Norwest Bank*, *supra*, 107 F.3d at 1307, this Court has addressed the question of the degree of conflict necessary to trigger the McCarran-Ferguson Act only in *SEC v. National Securities, Inc.*, 393 U.S. 453 (1969), and that decision involved federal and state statutes intended to protect entirely different interests.[5] The issue here presented has never been addressed by this Court.

The need for further guidance from this Court is apparent. The fact that four circuits have had to address the relationship between RICO and McCarran-Ferguson in the last two years itself attests to the significance of the question. It is also a clear predictor of the direction that future litigation will take in circuits that adopt the views of the Ninth Circuit. The attractiveness of RICO's remedies will assure that an ever increasing volume of insurance litigation will find its way to federal court for decision under federal law. Moreover, the issue presented is of broader reach than the specific question of whether McCarran-Ferguson precludes RICO claims. At issue is the question of whether the remedies provided by state laws regulating insurance are to be viewed as part of the substantive law of the state so as to preclude the application of *any* federal statute providing materially different remedies for the same conduct.

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[5] The First Circuit found a similar difference in the intended protections of Puerto Rican insurance law and the Federal Deposit Insurance Act. *Villafane-Neriz v. FDIC*, 75 F.3d 727, 736 (1st Cir. 1996).

That resolution of this issue should favor the Eighth, Sixth and Fourth Circuits, and not the Ninth Circuit's approach, is well argued in the Eighth Circuit's recent decision:

> "We find the latter line of cases more persuasive in the RICO context. As one court has noted, 'the remedies available under RICO are among the most severe ever enacted in a federal civil statute.' The state of Minnesota has determined that its insurance market can best be regulated by the Commissioner's pursuit of fines and injunctive relief. Congress has expressed its intention to leave the regulation of the business of insurance to the states unless a federal statute expressly addresses that subject or the application of a general federal statute would not invalidate, supersede, or impair a state statute. Were the question presented here, we might agree with the Sixth and Seventh Circuits that the federal civil rights statutes do not impair state insurance regulation. But Voyager [the insurance company defendant] makes a compelling case that the extraordinary remedies of RICO would frustrate, and perhaps even supplant, Minnesota's carefully developed scheme of regulation. We do not read the term 'impair' so narrowly as to permit the conclusion that the McCarran-Ferguson Act does not apply in the circumstances presented here." (Citations and footnote omitted.)

CONCLUSION

For the foregoing reasons, this petition for writ of certiorari should be granted.

August 18, 1997

Respectfully submitted

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Petitioners

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APPENDIX A

UNITED STATES COURT OF APPEALS
FOR THE NINTH CIRCUIT

No. 94-16548

Opinion of the Court of Appeals for the Ninth Circuit

(Decided May 23, 1997)

MARY FORSYTH; MARRIETTA CADE; WILLIE ANDREWS; MARY LOU BUEHLER; HELEN STAVES; RANDOLPH BRATTEN; and SEARLE AUTO GLASS, INC., d/b/a Best Glass Company,
Plaintiffs-Appellants,

v.

HUMANA, INC., a Delaware corporation; HUMANA HEALTH INSURANCE OF NEVADA, INC., a Nevada corporation; and DOES I through X, inclusive,
Defendants-Appellees.

May 23, 1997

Will Kemp, J. Randall Jones, Harrison, Kemp & Jones, Chtd., Las Vegas, NV, W.B. Markovits, Markovits & Greiwe, Cincinnati, Ohio, for the plaintiffs-appellants.

David N. Frederick, Las Vegas, Nevada and Robert N. Eccles, O'Melveny & Myers,

Washington D.C., for the defendants-appellees.

JUDGES: Before: J. Clifford Wallace and David R. Thompson, Circuit Judges, and Gordon Thompson, Jr., District Judge. * Opinion by Judge D. Thompson; Partial Concurrence and Partial Dissent by Judge Wallace.

* The Honorable Gordon Thompson, Jr., United States District Court Judge for the Southern District of California, sitting by designation.

THOMPSON, Circuit Judge:

There are two groups of plaintiffs in this case: (1) the employer purchasers of group health insurance policies issued by Humana Health Insurance of Nevada (Premium Payors); and (2) the employee beneficiaries of those policies who made coinsurance payments for health care received (Co-Payors).

The Co-Payors received hospital care from Humana Hospital-Sunrise (Sunrise Hospital), an acute care facility which is owned and operated by defendant Humana, Inc. Under its insurance agreements with the Co-Payors, Humana Health Insurance of Nevada (Humana Insurance) was obligated to pay 80% of the employees' hospital charges over and above a designated deductible amount; the Co-Payors were to pay the remaining 20%. Unknown to the plaintiffs, Humana Insurance negotiated a discount with Sunrise Hospital. Because of this discount, Humana Insurance ultimately paid significantly less than its 80% share of Sunrise Hospital's charges and the Co-Payors paid significantly more than their 20% co-payment share.

The Co-Payors contend Humana Insurance breached its contract with them, and violated the Employee Retirement

Income Security Act (ERISA), 29 U.S.C. § 1001, *et seq.*, by failing to pass along to them the Sunrise Hospital discounts in the form of reduced co-payments. The Co-Payors also contend the defendants violated ERISA by breaching fiduciary duties, engaging in prohibited transactions, and retaining excessive compensation. Both groups of plaintiffs contend that the defendants violated section 2 of the Sherman Act, 15 U.S.C. § 2, and engaged in a scheme to defraud in violation of the Racketeer Influenced and Corrupt Organizations Act (RICO), 18 U.S.C. §§ 1961-1968.

The district court granted summary judgment in favor of the defendants, rejecting all of the plaintiffs' claims except the Co-Payors' ERISA benefits breach of contract claim. This appeal followed.

We have jurisdiction under 28 U.S.C. § 1291. We affirm the district court's summary judgment on the ERISA claims, reverse the district court's summary judgment on the Sherman Act antitrust claims, and affirm in part and reverse in part summary judgment on the RICO claims.

I

FACTS

The plaintiffs contracted for health insurance, through employee benefit plans, with Humana Insurance during the period of 1985 through 1988. Pursuant to an agreement made in 1984 between Humana Insurance and Sunrise Hospital, Humana Insurance would receive a discount for its portion of the hospital charges incurred by its insureds. Unaware of this discount, the plaintiffs continued to pay

their required premiums and undiscounted co-payments. As a result, Humana Insurance ended up paying less than 80% of the hospital's charges for health care services, the Co-Payors paid more than 20% of these charges, and the Premium Payors paid the same premiums despite the reduced cost to Humana Insurance of the health care services.

The plaintiffs assert that Humana Insurance concealed the discount deal by writing checks to Sunrise Hospital for 80% of the billed charges. Sunrise Hospital would then remit the clandestine discount to Humana Insurance through monthly intercompany transfers, in what the plaintiffs characterize as a "classic kickback scheme."

II

PROCEDURAL HISTORY

The plaintiffs filed this action in the district court on March 29, 1989, alleging both state and federal claims. Six months later, the court granted the defendants' motion, pursuant to Federal Rule of Civil Procedure 12(b)(6), to dismiss the state law claims because those claims were preempted by ERISA.

The plaintiffs filed a first amended complaint on October 27, 1989. Following another round of motions, the district court upheld the sufficiency of the plaintiffs' ERISA claims, some of the RICO claims and the antitrust claims, and certified the two classes of plaintiffs, Premium Payors and Co-Payors. At the district court's direction, the plaintiffs filed a second amended complaint.

The second amended complaint asserted three claims for relief: (1) an ERISA claim by the Co-Payor class against Humana Insurance alleging breach of fiduciary duty, prohibited transactions and retention of excessive compensation; (2) an antitrust claim under section 2 of the Sherman Act by both classes of plaintiffs alleging the defendants had monopolized or attempted to monopolize the market for acute care facilities in Clark County, Nevada; and (3) a RICO claim by both classes of plaintiffs alleging Humana Insurance marketed and administered its policies through repeated acts of mail and wire fraud.

Again, the defendants moved for summary judgment. On July 21, 1993, after exhaustive briefing and argument, the district court granted the motion in its entirety ("the July 21, 1993 judgment"), but gave the plaintiffs leave to file a third amended complaint to assert on behalf of the Co-Payor class an ERISA benefits breach of contract claim against Humana Insurance under 29 U.S.C. § 1132(a)(1)(B).

The plaintiffs filed a third amended complaint. In that complaint they asserted only one claim, a breach of contract claim under ERISA in which they alleged that the Co-Payor class was entitled to benefits under 29 U.S.C. § 1132(a)(1)(B), because Humana Insurance had not properly allocated the discount it received from Sunrise Hospital.

The Co-Payor class moved for summary judgment on this claim; Humana Insurance also moved for summary judgment as to the proper measure of damages. The district court granted both motions on June 3, 1994 (the "June 3, 1994 judgment"), finding liability on the breach of contract claim, but limiting damages to the excess charges the

Co-Payor plaintiffs paid, and specifying the methodology for calculating the damages for these class members. Following further submissions by the parties, the district court entered an order on July 29, 1994 approving and adopting a schedule of damage awards by which the Co-Payors were to be reimbursed the amounts they had been overcharged on their co-payments.

On July 1, 1994, before the district court determined the damage awards, the plaintiffs filed a notice of appeal from the district court's June 3, 1994 judgment. The plaintiffs voluntarily dismissed that appeal after we determined that the June 3, 1994 judgment was not a final, appealable order. Thereafter, the plaintiffs filed a timely notice of appeal which brings this case before us.

The defendants have moved to dismiss this appeal in its entirety, or in the alternative to limit the appeal to the sole claim asserted in the plaintiffs' third amended complaint. In the defendants' words, their motion to dismiss raises two jurisdictional issues: "(1) whether plaintiffs' voluntary dismissal of an earlier appeal from the June 3[, 1994] Judgment precludes review of that judgment here; and (2) whether plaintiffs' failure to preserve in their Third Amended Complaint claims dismissed by the district court's earlier Order of July 21, 1993, precludes review of that ruling in this appeal."

On the merits, the defendants dispute the plaintiffs' various claims of error, and ask us to affirm the district court across the board. We first consider the motion to dismiss or limit the appeal.

III

MOTION TO DISMISS OR LIMIT APPEAL

The plaintiffs' voluntary dismissal of their premature appeal from the June 3, 1994 judgment does not preclude review of that judgment in this appeal. The June 3, 1994 judgment was not a final, appealable order when it was entered because the district court had not then determined the amount of damages. *See Brown v. United States Postal Serv.*, 862 F.2d 884, 886 (9th Cir. 1988). We deny the motion to dismiss the appeal in its entirety.

The defendants' contention that the appeal should be limited to the ERISA benefits breach of contract claim asserted in the third amended complaint presents a more complicated question. The plaintiffs did not reallege in that complaint any of the claims previously dismissed by the district court. The question is whether this omission precludes our review of the district court's dismissal of the previously asserted claims.

It is the law of this circuit that a plaintiff waives all claims alleged in a dismissed complaint which are not realleged in an amended complaint. *London v. Coopers & Lybrand*, 644 F.2d 811, 814 (9th Cir. 1981); *Sacramento Coca-Cola Bottling Co. v. Chauffeurs, Teamsters and Helpers Local 150*, 440 F.2d 1096, 1098 (9th Cir.), *cert. denied*, 404 U.S. 826, 30 L. Ed. 2d 54, 92 S. Ct. 57 (1971); *Loux v. Rhay*, 375 F.2d 55, 57 (9th Cir. 1967). This rule is premised on the notion that the "amended complaint supersedes the original, the latter being treated thereafter as non-existent." *Loux*, 375 F.2d at 57. If a plaintiff fails to

include dismissed claims in an amended complaint, the plaintiff is deemed to have waived any error in the ruling dismissing the prior complaint. *London*, 644 F.2d at 814; *Sacramento Coca-Cola*, 440 F.2d at 1098; *Loux*, 375 F.2d at 57.

We have declined, however, to extend the London rule to amended complaints that follow summary judgment. *See USS-POSCO Indust. v. Contra Costa County Bldg. & Const. Trades Council*, 31 F.3d 800, 812 (9th Cir. 1994). The rulings challenged by the plaintiffs in this appeal are rulings on summary judgment. As such, the claims which the court dismissed pursuant to these rulings have not been waived by the plaintiffs' failure to reallege them in the third amended complaint. *Id.* The defendants' motion to limit the issues in this appeal to the claim asserted in the plaintiffs' third amended complaint is therefore denied. To the extent, however, that the plaintiffs seek review of the district court's earlier dismissal of their state law claims pursuant to Federal Rule of Civil Procedure 12(b)(6), such claims were waived by the failure to reallege them after the Rule 12(b)(6) dismissal. *London*, 644 F.2d at 814.

IV

MERITS

A. Standard of Review

We review a grant of summary judgment de novo. *Warren v. City of Carlsbad*, 58 F.3d 439, 441 (9th Cir. 1995), *cert. denied*, 134 L. Ed. 2d 209, 116 S. Ct. 1261 (1996). We must determine, viewing the evidence in the light most favorable to the nonmoving party, whether there

are any genuine issues of material fact and whether the district court correctly applied the relevant substantive law. *Id.*

B. ERISA

The district court concluded the Co-Payor plaintiff class had no standing to bring their ERISA claim under 29 U.S.C. § 1132(a)(3) for breach of fiduciary duty. The court determined that the remedy provided by ERISA for the Co-Payors' harm was a benefits claim for breach of contract pursuant to section 1132(a)(1)(B) and not a claim for breach of fiduciary duty under section 1132(a)(3).

While this appeal was pending the Supreme Court issued its decision in *Varity Corp. v. Howe*, 134 L. Ed. 2d 130, 116 S. Ct. 1065 (1996). The Court held that an individual beneficiary may bring suit against a plan administrator for a breach of fiduciary duty under 29 U.S.C. § 1132(a)(3). Previous case law in this circuit had allowed such an action only in very limited circumstances not present in this case. *See, e.g., Amalgamated Clothing & Textile Workers Union v. Murdock*, 861 F.2d 1406 (9th Cir. 1988); *Waller v. Blue Cross of California*, 32 F.3d 1337 (9th Cir. 1994). The opinion in *Varity Corp.* expands what we previously viewed as permissible actions under section 1132(a)(3), but not to the extent necessary to accommodate the Co-Payors' claim in this case.

In *Varity Corp.* the Court emphasized that section 1132(a)(3) is a "catchall" provision which provides relief only for injuries that are not otherwise adequately provided for. 116 S. Ct. at 1078.

> Thus, we should expect that where Congress elsewhere provided adequate relief for a beneficiary's injury, there will likely be no need for further equitable relief, in which case such relief normally would not be "appropriate."

The Supreme Court thus interpreted the statute to allow individual relief for a breach of fiduciary duty in an ERISA action only where no other adequate relief is available.

The Court distinguished *Massachusetts Mut. Life Ins. v. Russell*, 473 U.S. 134, 144, 87 L. Ed. 2d 96, 105 S. Ct. 3085 (1985), which had interpreted section 1132(a)(2) to allow recovery for breach of fiduciary duty only where the relief is sought on behalf of the entire plan. Other than the fact that it had interpreted a different subsection of the statute, one of the factors which distinguished *Russell* was that another remedy, section 1132(a)(1), already provided specific relief for the sort of injury suffered in that case.

In the present case, the Co-Payors seek to recover individual relief under section 1132(a)(3) for Humana Insurance's breach of fiduciary duty. But the Co-Payors have already won a judgment for damages under section 1132(a)(1) for the injuries they suffered as a result of the defendant's actions. The district court determined, and we agree, that the Co-Payors are entitled to recover, in the form of damages pursuant to their claim under section 1132(a)(1), all amounts they were forced to pay over and above their contractual co-payment obligation.

In these circumstances, *Varity Corp.* does not authorize equitable relief under the catchall provision of section

1132(a)(3). Equitable relief under section 1132(a)(3) is not "appropriate" because section 1132(a)(1) provides an adequate remedy in this case.

We affirm the district court's summary judgment against the Co-Payors on their breach of fiduciary duty claim under section 1132(a)(3) of ERISA. We also affirm the district court's summary judgment in favor of the Co-Payors on their benefits breach of contract claim under section 1132(a)(1) of ERISA and the district court's computation of damages set forth in its July 29, 1994 order.

C. Antitrust

The plaintiffs sought approximately $ 181 million in damages against the defendants for monopolization and attempted monopolization under section 2 of the Sherman Act, 15 U.S.C. § 2.[1] The district court concluded the plaintiffs failed to produce evidence in support of this claim sufficient to survive a motion for summary judgment. Specifically, the district court determined the plaintiffs had

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[1] Title 15 U.S.C. @ 2 provides:

> Every person who shall monopolize, or attempt to monopolize, or combine or conspire with any other person or persons, to monopolize any part of the trade or commerce among the several States, or with foreign nations, shall be deemed guilty of a felony, and, on conviction thereof, shall be punished by fine not exceeding $ 10,000,000 if a corporation, or, if any other person, $ 350,000, or by imprisonment not exceeding three years, or by both said punishments, in the discretion of the court.

not provided sufficient evidence in support of their claim of monopolization to establish the relevant submarkets they asserted, the exercise of market power, or antitrust injury.

To prevail on a claim of monopolization under section 2 of the Sherman Act, a plaintiff must demonstrate: "(1) possession of monopoly power in the relevant submarket; (2) willful acquisition or maintenance of that power; and (3) causal antitrust injury." *Pacific Express, Inc. v. United Airlines*, Inc., 959 F.2d 814, 817 (9th Cir.), *cert. denied*, 506 U.S. 1034, 121 L. Ed. 2d 686, 113 S. Ct. 814 (1992).

Monopoly power, for the purpose of section 2 of the Sherman Act, is "the power to control prices or exclude competition." *United States v. Grinnell Corp.*, 384 U.S. 563, 571, 16 L. Ed. 2d 778, 86 S. Ct. 1698 (1966) (quoting *United States v. E. I. du Pont de Nemours & Co.*, 351 U.S. 377, 391, 100 L. Ed. 1264, 76 S. Ct. 994 (1956)). A plaintiff may demonstrate market power either by direct evidence or by circumstantial evidence. *Rebel Oil Co. v. Atlantic Richfield Co.*, 51 F.3d 1421, 1434 (9th Cir.), *cert. denied*, 133 L. Ed. 2d 424, 116 S. Ct. 515 (1995). Direct proof of market power may be shown by evidence of restricted output and supracompetitive prices. *Id.* Such a showing "is direct proof of the injury to competition which a competitor with market power may inflict, and thus, of the actual exercise of market power." *Id.*

The plaintiffs submitted evidence that Sunrise Hospital routinely charged higher prices than other hospitals while reaping high profits. With no accompanying showing of restricted output, however, the plaintiffs have failed to present direct evidence of market power.

The more common method of establishing monopoly power is by circumstantial evidence. This requires the plaintiff to "(1) define the relevant market, (2) show that the defendant owns a dominant share of that market, and (3) show that there are significant barriers to entry and show that existing competitors lack the capacity to increase their output in the short run."

It is impossible to determine market share without first defining the relevant market. *Id.* at 1434. Definition of the relevant market cannot be performed with mathematical accuracy; it is simply the recognition of a field in which meaningful competition is said to exist. *See United States v. Continental Can Co.*, 378 U.S. 441, 449, 12 L. Ed. 2d 953, 84 S. Ct. 1738 (1964). We have previously defined the relevant market as the group of sellers or producers who have the "actual or potential ability to deprive each other of significant levels of business." *Thurman Indus., Inc. v. Pay 'N Pak Stores, Inc.*, 875 F.2d 1369, 1374 (9th Cir. 1989). "The definition of the relevant market is a factual inquiry for the jury, and the court may not weigh evidence or judge witness credibility." *Rebel Oil*, 51 F.3d at 1435. *See also Thurman Indus.*, 875 F.2d at 1374.

The district court held the relevant market was the general acute care hospital market in Clark County, Nevada, which consisted of seven facilities in addition to Sunrise Hospital. The plaintiffs contend the district court arrived at this conclusion by weighing competing evidence. The plaintiffs argue the relevant market is smaller than the market found by the district court. They contend there are two distinct submarkets which the defendants monopolized

or attempted to monopolize: (1) the acute care hospitals actually used by Humana insureds; and (2) the major for-profit acute care hospitals in Clark County, Nevada. "A submarket exists if it is sufficiently insulated from the larger market so that supply and demand are inelastic with the larger market." *Morgan, Strand, Wheeler & Biggs v. Radiology, Ltd.*, 924 F.2d 1484, 1490 (9th Cir. 1991).

In support of their first contention that the relevant submarket is the acute care hospitals actually used by Humana insureds, the plaintiffs submitted an affidavit indicating that 75% to 85% of Humana insureds used Sunrise Hospital, and that this high proportion was achieved through contractual disincentives such as higher deductibles and co-payments for other hospitals, and noncontractual disincentives such as delaying or denying payment to other hospitals which made it difficult for Co-Payors touse them.

We reject the plaintiffs' attempt to limit the relevant market to acute care hospitals used by Humana insureds. The plaintiffs used Sunrise Hospital and obtained medical care from few other hospitals because of contractual provisions in their insurance policies. This tie-in defeats the plaintiffs' argument for a submarket consisting only of those hospitals Humana insureds actually used. *See Eastman Kodak Co. v. Image Technical Services, Inc.*, 504 U.S. 451, 490-91, 119 L. Ed. 2d 265, 112 S. Ct. 2072 (Scalia, J., dissenting) (1992). To succeed in the face of the contractual tie-in created by the insurance policies, the plaintiffs would have to make a showing of monopoly power in the health insurance market, and there is no evidence of this. *See id.*;

Jefferson Parish Hosp. Dist. 2 v. Hyde, 466 U.S. 2, 13-14, 80 L. Ed. 2d 2, 104 S. Ct. 1551 (1984).

The plaintiffs' second asserted relevant submarket consists of the major for-profit acute care hospitals in Clark County, Nevada. This submarket allegedly includes Sunrise Hospital, Valley Hospital, and Desert Springs Hospital. Excluded from this market are Clark County's five remaining acute care hospitals, Boulder City Hospital (Boulder City), Community Hospital of North Las Vegas (Community), St. Rose De Lima Hospital (St. Rose), University Medical Center (UMC), and Women's Hospital (Women's). The plaintiffs submitted evidence tending to support their contention that the five remaining acute-care hospitals in Clark County, which the district court included in its determination of the relevant market, were not competitors of Sunrise Hospital. For example, there was evidence that UMC charged patients 15 to 20 percent less than Sunrise Hospital. Yet, because UMC was considered a hospital for the indigent, patients with a choice, such as those with insurance, eschewed UMC in favor of the larger for-profit hospitals despite the significantly lower price at UMC.

We recognize "the scope of the relevant market is not governed by the presence of a price differential." *Twin City Sportserv., Inc. v. Charles O. Finley & Co.*, 512 F.2d 1264, 1274 (9th Cir. 1975). But when demand for the commodity of one producer shows no relation to the price for the commodity of another producer, it supports the claim that the two commodities are not in the same relevant market.

There was also evidence that Boulder City, St. Rose, Community, and Women's were considered "niche" hospitals that did not offer the range of services available at Sunrise Hospital. This, alone, is not enough. Specialty shops which offer only a limited range of goods are generally considered in the same market with larger, more diverse, "one-stop shopping" centers. *Thurman Indust.*, 875 F.2d at 1374-77. However, the plaintiffs also submitted documentary evidence, including an expert's affidavit and testimony before the Nevada legislature, which tended to support their contention that these specialty hospitals did not, in fact, compete with Sunrise Hospital.

We have not recounted the evidence submitted by the defendants, but it was substantial. Were we to weigh the evidence we might be inclined to find that the plaintiffs failed to establish their alleged submarket. On a motion for summary judgment, however, this would be inappropriate.

We conclude the evidence the plaintiffs presented was sufficient to withstand the defendants' motion for summary judgment as to the plaintiffs' second asserted relevant market.

The additional issues of monopoly power and the maintenance of monopoly power depend upon a resolution of the relevant market question. *Rebel Oil*, 51 F.3d at 1434. Whether the defendants engaged in anticompetitive behavior is dependent upon resolution of these issues. *See Pacific Express*, 959 F.2d at 818.

The same may be said of the plaintiffs' section 2 claim for attempted monopolization. To prevail on such a claim, a

plaintiff must demonstrate four elements: (1) specific intent to control prices or destroy competition; (2) predatory or anticompetitive conduct directed toward accomplishing that purpose; (3) a dangerous probability of success; and (4) causal antitrust injury. *Rebel Oil*, 51 F.3d at 1433.

The plaintiffs' claim of attempted monopolization was not directly addressed by the district court. Resolution of this issue, like the claim of actual monopolization, is dependent upon a definition of the relevant market. Without such a determination, we cannot assess whether challenged activity was anticompetitive. In sum, the determination of all these subsidiary issues depends on the determination of the question of the relevant market; and there is a genuine dispute of material fact as to that.

The plaintiffs should also survive summary judgment on the issue of causal antitrust injury. To survive summary judgment, the plaintiffs were required to offer some evidence demonstrating "the existence of an 'antitrust injury, which is to say injury of the type the antitrust laws were intended to prevent and that flows from that which makes defendant's acts unlawful.'" *Atlantic Richfield v. USA Petroleum*, 495 U.S. 328, 334, 109 L. Ed. 2d 333, 110 S. Ct. 1884 (1990) (quoting *Brunswick Corp. v. Pueblo Bowl-O-Mat*, 429 U.S. 477, 489, 50 L. Ed. 2d 701, 97 S. Ct. 690 (1977)).

Here, the plaintiffs presented evidence that, as a result of Humana's anticompetitive behavior, they paid higher copayments and premiums than they would have paid in a competitive market. This evidence, while it shows that the plaintiffs paid high copayments and premium payments because of the kickback scheme, does not explain how the

scheme reduced competition in the relevant market. We conclude, therefore, that the plaintiffs did not demonstrate that they suffered antitrust injury resulting from the kickback scheme. *See Atlantic Richfield*, 495 U.S. at 344 (observing that "the antitrust injury requirement ensures that a plaintiff can recover only if the loss stems from a competition-reducing aspect or effect of the defendant's behavior.").

The plaintiffs presented other evidence, however, that Humana engaged in impermissibly anticompetitive conduct in three ways: (a) Sunrise (the Humana-owned hospital) was the only area hospital certified to provide Level III neonatal care, but refused to provide Level III care under separate contract; (b) Sunrise diverted indigent or low-paying critical care patients to other area hospitals; and (c) Humana threatened physicians who did not support its monopoly.

This conduct describes antitrust violations. The plaintiffs presented evidence that Sunrise required customers who wanted to buy Level III neonatal care to also buy other neonatal services. Such a practice could constitute an anticompetitive tying arrangement. *See Datagate, Inc. v. Hewlett-Packard Co.*, 60 F.3d 1421, 1423 (9th Cir. 1995) (describing a tying arrangement as a "device used by a competitor with market power in one market . . . to extend its market power into an entirely distinct market."). Sunrise's alleged policy of funneling indigent and low-paying patients to competitors raises a factual question whether such conduct increased the operating cost of those competitors by imposing on them the cost of caring for indigent patients. *See Multistate Legal Studies, Inc. v. Harcourt Brace*

Jovanovich Legal and Professional Publications, Inc., 63 F.3d 1540, 1553 (10th Cir. 1995) (stating that monopolist's practice of scheduling courses to conflict with competitor's courses could raise competitor's costs, and therefore, "would qualify as anticompetitive conduct"); *Premier Electrical Constr. Co. v. Nat'l Electrical Contractors Ass'n*, 814 F.2d 358, 368 (7th Cir. 1987) (observing that, when defendant "raised its rivals' costs," it "raised the market price to its own advantage."); *see generally* T. Krattenmaker & S. Salop, Anticompetitive Exclusion: Raising Rivals' Costs to Achieve Power Over Price, 96 Yale L.J. 209, 235-262 (1986). Finally, the plaintiffs presented evidence that Humana's alleged practice of threatening physicians who disagreed with their monopoly limited competition by preventing physicians from referring patients to hospitals other than Sunrise. *See Potters Medical Center v. City Hospital Ass'n*, 800 F.2d 568, 575 (6th Cir. 1986) (holding that hospital's alleged conduct in restricting privileges of physicians "could state a monopolization claim.").

According to the plaintiffs' evidence, these anticompetitive practices enabled Sunrise to, and it did, charge higher prices for hospital services. These higher prices translated into higher copayments and premium payments. Such an increase in consumer prices caused by the asserted conduct would constitute antitrust injury of the type the antitrust laws were designed to prevent. *See Blue Shield of Virginia v. McCready*, 457 U.S. 465, 73 L. Ed. 2d 149, 102 S. Ct. 2540 (1982) (holding that individual health care plan subscriber could sue health care provider for antitrust injury resulting from provider's alleged boycott of psychologists); *Reiter v. Sonotone Corp.*, 442 U.S. 330,

341, 60 L. Ed. 2d 931, 99 S. Ct. 2326 (1979) (holding that consumers of retail goods and services have standing to sue under the antitrust laws when they suffer a price increase resulting from anticompetitive conduct).

We conclude, therefore, that the plaintiffs are entitled to survive summary judgment on the issue of antitrust injury. In calculating the plaintiffs' antitrust damages, the jury will be faced with the difficult task of separating the antitrust damages from the damages resulting from Humana's kickback scheme. Complex antitrust cases, however, invariably involve complicated questions of causation and damages. *See Potters Medical Center*, 800 F.2d at 576 (observing that speculative nature of damages are "often the case in complex antitrust litigation and should not in itself foreclose antitrust standing.").

We reverse the district court's summary judgment in favor of the defendants on the plaintiffs' antitrust claim.

D. RICO

Both the Premium Payor and the Co-Payor classes alleged that during the period from 1985 through 1988, in violation of the civil RICO statute, 18 U.S.C. § 1964(e):[2]

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[2] Title 18 U.S.C. @ 1964(c) provides:

Any person injured in his business or property by reason of a violation of section 1962 of this chapter may sue therefor in any appropriate United States district court and shall recover threefold the damages he sustains and the cost of the suit, including a reasonable attorney's fee.

(1) Humana, Inc. and Humana Insurance acted as members of an "association in fact" enterprise, or alternatively, that Humana, Inc., a person within the meaning of 18 U.S.C.§ 1961(3), associated with Humana Insurance, an enterprise within the meaning of 18 U.S.C. § 1961(4); (2) Humana Inc. and Humana Insurance entered into a secret agreement to give Humana Insurance an excessive discount for hospital charges incurred by its insureds; (3) Humana Inc. and Humana Insurance concealed and misrepresented this agreement in numerous mailings, television and radio commercials, and telephone calls; and (4) such acts were intended to defraud the Premium Payor class into purchasing policies and the Co-Payor class into paying excessive co-payments.

The district court granted the defendants' motion for summary judgment because it found the plaintiffs' RICO claims were barred by the McCarran-Ferguson Act. Additionally, the district court determined that the Premium Payors' RICO claim failed because the Premium Payors failed to demonstrate a direct relationship between the alleged injurious conduct and the injury asserted (excessive premiums), or any "'concrete financial injury,' for the purposes of RICO." The district court also held the defendants were "entitled to summary judgment on the Co-Payor class's RICO claim to the extent that the Co-Payor class [sought] damages in excess of the amount that could have been recovered under a proper interpretation of the co-payment provision."

1. The McCarran-Ferguson Act

The MFA, 15 U.S.C. § 1011, *et seq.*, was enacted, in part, to allow states to regulate the business of insurance free from inadvertent preemption by federal statutes of general applicability. *Merchants Home Delivery Serv., Inc. v. Frank B. Hall & Co.*, 50 F.3d 1486, 1488-89 (9th Cir.), *cert. denied*, 116 S. Ct. 418, 133 L. Ed. 2d 335 (1995). Section 2(b) of the MFA provides in pertinent part:

> No Act of Congress shall be construed to invalidate, impair, or supersede any law enacted by any State for the purpose of regulating the business of insurance . . . unless such Act specifically relates to the business of insurance.

We have adopted a four-part test to determine when section 2(b) of the MFA precludes the application of a federal statute to preempt state insurance law. *Merchants Home*, 50 F.3d at 1489. The MFA precludes such preemption by a federal statute if: "(1) the statute does not 'specifically relate' to the business of insurance, (2) the acts challenged under the statute constitute the business of insurance, (3) the state has enacted a law or laws regulating the challenged acts, and (4) the state law would be superseded, impaired or invalidated by the application of the federal statute." *Id.* All four factors must be satisfied. *Id.* Only the first factor is not disputed in this case - the parties concede RICO does not specifically relate to the business of insurance.

With regard to the second factor, the crux of the plaintiffs' claim is that Humana Insurance misrepresented

how premiums would be calculated, leading the Premium Payors to believe a discount from Sunrise Hospital would lower their premium rates, and that the defendants overcharged the Co-Payors by billing them for 20% of the undiscounted cost of their hospital care.

The plaintiffs argue these allegedly fraudulent acts cannot be part of the business of insurance because "fraud is not a legitimate aspect of risk management." In *Merchants Home* we rejected such an argument and we do so again. It is useless "to point to a practice forbidden by federal law . . . and observe that this practice is not itself insurance." *Id.* at 1490 (quoting *NAACP v. American Family Mut. Ins. Co.*, 978 F.2d 287, 294 (7th Cir. 1992), *cert. denied*, 508 U.S. 907, 124 L. Ed. 2d 247, 113 S. Ct. 2335 (1993)). Such an interpretation would "read the McCarran-Ferguson Act out of existence" because "any practice which violated any federal statute would, by definition, not be the 'business of insurance,' resulting in all federal statutes applying to the business of insurance with their 'full rigor.'" *Id.*

The acts the plaintiffs challenge are analytically equivalent to the overcharging for premiums alleged in Merchants Home. There, the plaintiff asserted RICO claims for fraudulent insurance practices by the defendant. *Id.* at 1488. The plaintiff alleged three types of fraudulent practices: (1) overcharging for premiums on actual policies; (2) collecting premiums on fictitious policies; and (3) collecting money from the insured to pay uninsured claims which the insurer did not actually pay. *Id.* We held overcharging for premiums was part of the "business of insurance," though collection of premiums on nonexistent

policies and collecting money from the insured for bogus uninsured claims were not part of such business. *Id.* at 1490.

Consistent with Merchants Home, we conclude the challenged practices in this case of Humana Insurance overcharging premiums and not reducing co-payments is the "business of insurance" for the purpose of the MFA.

The third factor in the preemption analysis is whether the state has enacted a law or laws regulating the challenged acts. We agree with the district court that the Nevada Act was enacted to regulate the acts challenged by the plaintiffs.

In 1971, Nevada adopted the Trade Practices and Frauds; Financing of Premiums Act (Act), N.R.S. § 686A.010, *et seq*. The purpose of the Act was:

> to regulate trade practices in the business of insurance in accordance with the intent of Congress as expressed in the [MFA], 15 U.S.C. §§ 1011 to 1015 inclusive, by defining, or providing for the determination of, all such practices in this state which constitute unfair methods of competition or unfair or deceptive acts or practices and by prohibiting the trade practices so defined or determined.

N.R.S. § 686A.010.

Under the Act, the Nevada Commissioner of Insurance is granted "exclusive jurisdiction" to regulate the trade practices of the business of insurance. N.R.S. § 686A.015. The Act specifically prohibits the dissemination of false or misleading information relating to the sale of policies or benefits. N.R.S. §§ 686A.030 & 686A.040. The plaintiffs'

observation that the Act provides for only administrative remedies does not alter our finding that it regulates the challenged acts. The third factor is satisfied.

Applying the fourth factor, we consider whether the application of RICO would invalidate, impair, or otherwise supersede Nevada's legislation. In *Merchants Home*, we addressed, for the first time, the question whether a federal statute invalidated, impaired, or superseded a state law under section 2(b) of the MFA. *Merchants Home*, 50 F.3d at 1492. As in the present case, the state law in Merchants Home provided for only administrative remedies, but the federal law (RICO) provided for a private right of action. *Id.* We held the application of the federal statute, although it "prohibited" acts which were also prohibited under the state's insurance law, did "not 'invalidate, impair, or supersede' the state's laws under § 2(b) of the McCarran-Ferguson Act." *Id.*

Here, as in *Merchants Home*, there is some symmetry between RICO's private right of action and Nevada's administrative scheme. RICO permits an injured plaintiff to recover for fraudulent practices which the Nevada administrative scheme proscribes. This symmetry, however, does not create a conflict between federal and state law. *Id.* Accordingly, the fourth factor is not present. The MFA does not preclude the plaintiffs' RICO claims.[3]

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[3] The district court cannot be faulted for ruling to the contrary. It did not have the benefit of Merchants Home, decided in 1995, when it rendered its summary judgment in this case.

The next question we consider is whether the plaintiffs produced enough evidence in support of their RICO claims to survive summary judgment.

2. Premium Payors' Claim

To state a claim under RICO, 18 U.S.C. § 1962(c), a plaintiff must demonstrate: (1) the conduct; (2) of an enterprise; (3) through a pattern; (4) of racketeering activity.[4] *Sun Sav. and Loan Ass'n v. Dierdorff*, 825 F.2d 187, 191 (9th Cir. 1987). Racketeering activity is any act indictable under various provisions of 18 U.S.C. § 1961 and includes the predicate acts alleged in this case of mail fraud and wire fraud under 18 U.S.C. §§ 1341 and 1343. *Id.* Mail fraud under section 1341 requires the showing of a scheme to defraud involving use of the United States mails with the specific intent to defraud. *Schreiber Distributing Co. v. Serv-Well Furniture Co.*, 806 F.2d 1393, 1399-1400 (9th Cir. 1986). Wire fraud under section 1343 requires the same showing, but involves use of United States wires. *Id.* at 1400.

The district court held the Premium Payor class failed to demonstrate the requisite causal connection between the

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[4] Title 18 U.S.C. @ 1962(c) provides:

> It shall be unlawful for any person employed by or associated with any enterprise engaged in, or the activities of which affect, interstate or foreign commerce, to conduct or participate, directly or indirectly, in the conduct of such enterprise's affairs through a pattern of racketeering activity or collection of unlawful debt.

alleged predicate acts and the asserted damages. The court also held the class failed to demonstrate "concrete financial injury."

To maintain a claim under RICO, a plaintiff must show not only that the defendant's violation was a "but for" cause of his injury, but that it was the proximate cause as well. *Imagineering, Inc. v. Kiewit Pacific Co.*, 976 F.2d 1303, 1311 (9th Cir. 1992), *cert. denied*, 507 U.S. 1004, 123 L. Ed. 2d 266, 113 S. Ct. 1644 (1993). This requires a showing of a direct relationship between the injurious conduct alleged and the injury asserted. *Id.* The plaintiff must show a concrete financial loss. *Id.*

In the present case, the Premium Payor class had to establish that they relied on misrepresentations in buying their insurance policies, and that these misrepresentations directly caused them a concrete financial loss. They contend their claim is simple. Humana, Inc. and Humana Insurance represented to them, through mailings, television and radio commercials, and phone calls, that cost savings would be passed along to them in the form of reduced premiums. They relied on these representations in choosing Humana Insurance as their insurance carrier. The hospital discount Humana Insurance received from Sunrise Hospital was a form of cost saving, yet it was not incorporated into calculation of their premiums. Because their premiums did not reflect these discounts, the Premium Payors contend they suffered a financial loss.

The district court found no evidence in the record to support the Premium Payors' contention that the defendants fraudulently represented how premiums would be calculated.

The class argues the myriad advertisements and promotions did just that. We disagree. The transcript of commercials submitted by the class fails to support their argument. The commercials simply state that because Humana Insurance can "control costs" the employers and employees can save money. The district court accurately described such statements as "puffery." The statements were too general to be interpreted as defining any calculation of insurance premiums.

The class also submitted Humana Insurance's annual reports. Statements in the reports suggested patients were encouraged to attend Humana hospitals "where savings could be shared with employers and their employees through lower premiums." Like the advertisements, this statement did not make a specific representation as to how the premiums would be calculated sufficient to support a cause of action for fraud.

Because we conclude the Premium Payor class failed to come forward with any evidence that the defendants misrepresented how the insurance premiums would be calculated, we do not reach the question of causation. The district court did not err in granting summary judgment against the Premium Payors on their RICO claim.

3. Co-Payors' Claim

The Co-Payors alleged the failure to disclose the Sunrise Hospital discount in the notice of coinsurance obligations sent to them induced them to make inflated co-payments. The Co-Payors were thus allegedly injured by the

defendants' fraudulent failure to reduce their co-payment bills in accordance with the agreed ratio of 80% to 20%.

Although these facts presented at least a triable issue of fact on the Co-Payors' RICO claim, the district court dismissed the claim as barred by the McCarran-Ferguson Act. As we have previously stated, the McCarran-Ferguson Act does not preclude a RICO claim in this case. The district court correctly held, however, that the Co-Payors failed to present any evidence of financial harm resulting from the misrepresentations, except for the amounts they overpaid as a result of the nondisclosure of the discount. Therefore, we affirm the district court's partial summary judgment in favor of the defendants as to any claim by the plaintiffs for RICO damages in excess of these overpayments.

E. Motion To Amend

The plaintiffs contend the district court erred in not granting them leave to amend their complaint. The plaintiffs sought leave to add three additional claims: (1) a RICO conspiracy claim; (2) a claim for obstruction of justice under 18 U.S.C. § 1503; and (3) their previously dismissed state law claims.

We review for abuse of discretion a district court's denial of a motion to amend. *DCD Programs, Ltd. v. Leighton*, 833 F.2d 183, 186 (9th Cir. 1987).

Leave to amend pleadings "shall be freely given when justice so requires." Fed. R. Civ. P. 15(a). We apply this rule with "extreme liberality." *Morongo Band of Mission Indians v. Rose*, 893 F.2d 1074, 1079 (9th Cir. 1990). The following factors guide a court's determination of whether a

motion to amend should be granted: (1) undue delay; (2) bad faith; (3) futility of amendment; and (4) prejudice to the opposing party. *Hurn v. Retirement Fund Trust of Plumbing, Etc.*, 648 F.2d 1252, 1254 (9th Cir. 1981). Of these elements, only the futility issue is relevant in this case.

Because the plaintiffs' state law claims are preempted by ERISA, leave to amend to reassert the state law claims would be futile. The obstruction of justice claim under 18 U.S.C. § 1503 is also futile because 18 U.S.C. § 1503 is a criminal statute that does not provide for a private cause of action. *Hanna v. Home Ins. Co.*, 281 F.2d 298, 303 (5th Cir. 1960), *cert. denied*, 365 U.S. 838, 5 L. Ed. 2d 747, 81 S. Ct. 751 (1961); *Odell v. Humble Oil & Refining Co.*, 201 F.2d 123, 127 (10th Cir.), *cert. denied*, 345 U.S. 941, 97 L. Ed. 1367, 73 S. Ct. 833 (1953). With regard to the proffered RICO conspiracy claim, however, in light of our determination that the Co-Payor class's RICO claim is not barred by the MFA, we conclude the Co-Payor class should be allowed to amend its complaint to assert this claim.

V

CONCLUSION

The district court's grant of summary judgment is affirmed as to the ERISA issues, reversed and remanded as to the Sherman Act antitrust issues, affirmed as to the Premium Payor class's RICO claim, and affirmed as to partial summary judgment on the Co-Payor class's RICO claim. In addition, we grant the Co-Payor class leave to amend its RICO claim to allege a RICO conspiracy. The

plaintiffs shall recover from the defendants one-half of the plaintiffs' cost of appeal.

AFFIRMED IN PART, REVERSED IN PART, AND REMANDED.

WALLACE, Circuit Judge, concurring and dissenting:

I concur with the majority in large measure, but its analysis of the antitrust claims in part IV.C. is so troubling that I cannot agree. Although reasonable minds can differ, in this complex economic terrain, I am inclined to the position that the district court did not err in concluding that Forsyth's evidence for a submarket of for-profit hospitals was insufficient to survive summary judgment. The majority's analysis of this issue displays a subtle but important error of market definition theory. I would affirm.

Forsyth contends that Humana monopolized or attempted to monopolize a market for major for-profit acute care hospitals in Clark County, Nevada. The district court held that Forsyth failed to establish the existence of a separate market for for-profit hospitals within the general Clark County hospital market. In holding that there is a genuine factual dispute on this issue, the majority points to two categories of evidence submitted by Forsyth. The first depends on a faulty inference about cross-price elasticity of demand, and the second is insufficient to avoid summary judgment.

As evidence for the existence of a separate market, the majority states that although "UMC charged patients 15 to 20 percent less than Sunrise Hospital, . . . patients with a choice, such as those with insurance, eschewed UMC in

favor of the larger for-profit hospitals despite the significantly lower price at UMC." The majority interprets this consumer behavior as evidence that demand for for-profit hospitals is not responsive to the price charged by non-profit hospitals; "when demand for the commodity of one producer shows no relation to the price for the commodity of another producer, it supports the claim that the two commodities are not in the same relevant market."

The majority's test refers to the economic concept of cross-price elasticity of demand, which is a useful tool for evaluating whether products compete in the same market. Cross elasticity of demand measures the percentage change in the quantity which consumers demand of one product in response to a percentage change in the price of another.

> When the cross-price elasticity of demand for good i with respect to the price of good j is positive, an increase in [the price of] j causes the demand for good i to rise; in that case, the goods are substitutes (as fuel oil and coal). Conversely, if the cross-price elasticity is negative, the goods are complements (as cigars and matches).

2A Phillip E. Areeda, et al., Antitrust Law 105 (rev. ed. 1995). A high cross elasticity of demand indicates that products are close substitutes, and should probably be treated as part of the same market. A low or zero cross elasticity of demand is evidence that products do not compete in the same relevant market.

By focusing on this relationship between the demand for one commodity and the price of another, the majority

attempts to avoid the common but fallacious assumption that expensive products automatically compete in a separate market from inexpensive ones. *See Twin City Sportservice v. Charles O. Finley & Co.*, 512 F.2d 1264, 1274 (9th Cir. 1975) ("the scope of the relevant market is not governed by the presence of a price differential"). The majority reasons that if "patients with a choice" prefer Sunrise hospital even though it is more expensive, then demand for Sunrise "shows no relation" to the prices charged at UMC and the other "indigent" hospitals.

That inference causes me to depart from the majority. We cannot conclude that demand for Sunrise "shows no relation" to the prices at UMC without some evidence of how demand for Sunrise responds to a change in the prices at UMC. Cross elasticity of demand measures a dynamic relationship between two variables: the quantity demanded of product i, and the price of product j. That is, by economic formulation,

$$Eij\ d = - \frac{(\text{change in})\ Qi\ d\ /Qi\ d}{(\text{change in})\ Pj\ /Pj}$$

2A Areeda, Antitrust Law at 105. Cross elasticity cannot be calculated or approximated without information about the quantity demanded of i at (at least) two different prices of j. The record does not reflect that Forsyth introduced any such evidence, and without it the majority's inference is inappropriate.

The following example may help in making my point. Suppose that Ford and Chevrolet each sell a light truck. The Ford has four-wheel drive, and costs $ 2000 more than the Chevrolet, which does not have four-wheel drive. Both trucks sell equally well. If Chevrolet cuts the price of its truck in half and Ford's sales are unaffected, we might infer that the two vehicles are in different relevant markets. But the majority would interpret the simple fact that some consumers buy the Ford, notwithstanding the price differential, as evidence that the trucks do not compete. That reasoning, it seems to me, is faulty.

If four-wheel drive is worth about $ 2000 to most consumers, for example, small changes in the price of the Ford could have a dramatic impact on demand for the Chevrolet. If the availability of the Chevrolet significantly limits the rational pricing strategies for Ford, then the two trucks "compete" in the sense that matters for antitrust market definition. Ford could not exact monopoly profits by raising the price of its truck, because it would lose too many customers to Chevrolet. *See Rebel Oil v. Atlantic Richfield*, 51 F.3d 1421, 1434 (9th Cir.) ("If the sales of other producers substantially constrain the price-increasing ability of the monopolist or hypothetical cartel, these other producers must be included in the market."), *cert. denied*, 133 L. Ed. 2d 424, 116 S. Ct. 515 (1995).

The majority also points to certain documentary evidence, including an expert's affidavit and testimony before the Nevada legislature, which tended to support Forsyth's contention that the "niche" hospitals did not compete with Sunrise. "Niche" sellers are generally

considered to be in the same market with larger, more diverse vendors which carry the same products. *See Thurman Industries v. Pay 'N Pak Stores*, 875 F.2d 1369, 1374-77 (9th Cir. 1989). I do not believe that the district judge erred in concluding that Forsyth's evidence on this point was insufficient to create a triable issue of fact.

APPENDIX B

Memorandum Decision and Order of the District Court
(Decided July 21, 1993)

MARY FORSYTH, MARRIETTA CADE; WILLIE ANDREWS; MARY LOU BUEHLER; HELEN STAVES; RANDOLPH BRATTON; and SEARLE AUTO GLASS, INC., doing business as BEST GLASS COMPANY,
Plaintiffs,

v.

HUMANA, INC., a Delaware corporation; HUMANA HEALTH INSURANCE OF NEVADA, INC., a Nevada corporation; and DOES I through X, inclusive, Defendants.

CV-S-89-249-PMP (LRL)

UNITED STATES DISTRICT COURT
FOR THE DISTRICT OF NEVADA

July 21, 1993, Decided

J. Randall Jones, John Field, Jones, Jones, Close & Brown, Las Vegas, NV, Will Kemp, Harrison Kemp, Chtd., Las Vegas, NV, Stanley M. Chesley, Waite, Schneider, Bayless & Chelley, L.P.A., Cincinnati, OH, John J. Cummings, Richard M. Martin, Jr., W.B. Markovitis, Cummings, Cummings & Dudenhefer, New Orleans, LA, Wendell H. Gauthier, Gauthier & Murphy, Metairie, LA, B. Robert Blakey, Notre Dame Law School, Notre Dame, IN, for plaintiffs.

Dennis Kennedy, Louis Garfinkel, Lionel Sawyer & Collins, Las Vegas, NV, Mark Biros, Joseph E. Casson, Michael F. Moses, Thomas H. Brock, Burton J. Fishman, Lisolette Mitz, Proskauer Rose Goetz & Mendelson, Washington, D.C., Robert Eccles, O'Melveny & Myers, Washington, D.C., for defendants.

PRO, District Judge

Plaintiffs are employers and employees who contracted for health insurance, through employee benefit plans, with Defendant Humana Health Insurance of Nevada, Inc. ("Humana Insurance"), during the period 1985 to 1988. One group of Plaintiffs used the health care services of Humana Hospital-Sunrise ("Sunrise Hospital"), an acute care facility located in Las Vegas, Nevada, which is owned and operated by Defendant Humana, Inc. ("Humana"), and a Participating Hospital under the insurance agreements. The insurance agreements required those insured with Humana Insurance to pay all expenses up to the designated deductible amount, and 20% of the expenses beyond that, with the insurance company to pay the other 80%. The liability of an insured had a cap called a "personal expense limit" which was the maximum the insured would have to pay in any given year, regardless of the total health care expenses.

In 1984, Humana Insurance and Sunrise Hospital entered into an agreement whereby Humana Insurance would pay Sunrise Hospital a discounted amount for that portion of the hospital charges for which it was responsible. However, the portion of the charges paid by the insureds was not discounted but was still based on the usual and customary rates. Plaintiffs assert that Humana Insurance failed to pass

along the discounts it had arranged for itself to its insureds either in the form of reduced premiums or reduced co-payments.

Previous Orders of this Court have granted Plaintiffs' Motions for Class Certification, thereby permitting both a Premium Payor Class and a Co-Payor Class to maintain this action. By their Second Amended Complaint (#370), filed August 12, 1991, Plaintiffs assert three claims for relief. Plaintiffs' First Claim for Relief is brought by the Co-Payor Class which consists of employees who obtained health insurance benefits under an employee benefit plan as defined under the Employment Retirement Income Security Act (ERISA), 29 U.S.C. § 1001, *et seq*. There Plaintiffs allege that Defendants breached fiduciary duties owed to the Co-Payor Class under ERISA; engaged in transactions prohibited by ERISA; and retained excessive compensation. Plaintiffs' Second Claim for Relief alleges a violation of Section 2 of the Sherman Act, 15 U.S.C. § 2 and is brought on behalf of the Co-Payor Class and Premium Payor Class, which consists of individuals or entities paying all or a portion of the Humana-Insurance premiums during the period 1985 to 1988. Plaintiffs' Third Claim for Relief, brought on behalf of both the Co-Payor Class and Premium Payor Class, alleges that Defendants engaged in a scheme to defraud in violation of the Racketeer Influenced and Corrupt Organizations Act, 18 U.S.C. §§ 1961-1968.

Before the Court is a Motion for Summary Judgment (#804), filed on June 22, 1993, by Defendants Humana and Humana Insurance. On July 7, 1992, Defendants filed a Correction (#808). Plaintiffs filed their Opposition (#820) on

August 7, 1992, which was followed by an Errata (#822) filed on August 12, 1992. Defendants filed their Reply (#841) accompanied by a Supplemental Affidavit (#842) on October 19, 1992.

Additionally, the parties have filed the following supplementary motions:

(1) supplement to Plaintiffs' Opposition (#834), filed on September 30, 1992.

(2) Second supplement to Plaintiffs' Opposition (#849), filed on November 16, 1992.

(3) Defendants' Supplemental Memorandum (#868), filed on February 22, 1993. Plaintiffs filed a Third Supplement and Opposition (#890) on May 17, 1993. On June 2, 1993, Defendants filed a Supplemental Memorandum (#895) in response.

(4) Defendants' Second Supplemental Memorandum (#891), filed on May 21, 1993.

(5) Plaintiffs' Fourth Supplement to Opposition (#896), filed on June 8, 1993. Defendants filed a Response (#909), on July 1, 1993.

(6) Defendants' Third Supplemental Memorandum (#901), filed on June 23, 1993. Plaintiffs filed a Response to Defendants' Second and Third Supplemental Memoranda (#908) on July 1, 1993.

(7) Plaintiffs' Response to (#895) and Fifth Supplement to Opposition (#902), filed on June 25, 1993.

(8) Defendants' Supplemental Memorandum of Points and Authorities (#903), filed on June 29, 1993.

(9) Defendants' Fourth Supplemental Memorandum (#910), filed on July 2, 1993.

On July 7, 1993, this Court heard oral argument on Defendants' Motion for Summary Judgment and its progeny.

I. STANDARD OF REVIEW

Pursuant to Federal Rule of Civil Procedure 56, summary judgment is proper "if the pleadings, depositions, answers to interrogatories, and admissions on file, together with the affidavits, if any, show that there is no genuine issue as to any material fact and that the moving party is entitled to a judgment as a matter of law.

The party moving for summary judgment has the initial burden of showing the absence of a genuine issue of material fact. *See Adickes v. S. H. Kress & Co.*, 398 U.S.144, 26 L. Ed. 2d 142, 90 S. Ct. 1598 (1970); *Zoslaw v. MCA Distrib. Corp.*, 693 F.2d 870, 883 (9th Cir. 1982). Once the movant's burden is met by presenting evidence which, if uncontroverted, would entitle the movant to a directed verdict at trial, the burden then shifts to the respondent to set forth specific facts demonstrating that there is a genuine issue for trial. *Anderson v. Liberty Lobby, Inc.*, 477 U.S. 242, 250, 91 L. Ed. 2d 202, 106 S. Ct. 2505 (1986). If the factual context makes the respondent's claim implausible, that party must come forward with more persuasive evidence than would otherwise be necessary to show that there is a genuine issue for trial. *Celotex Corp. v. Catrett*, 477 U.S.

317, 323-24, 91 L. Ed. 2d 265, 106 S. Ct. 2548 (1986); *Matsushita Elec. Indus. Co. v. Zenith Radio Corp.*, 475 U.S. 574, 586-87, 106 S. Ct. 1348, 89 L. Ed. 2d 538 (1986); *California Arch. Bldg. Prods. v. Franciscan Ceramics*, 818 F.2d 1466, 1468 (9th Cir. 1987), *cert. denied*, 484 U.S. 1006, 98 L. Ed. 2d 650, 108 S. Ct. 698 (1988).

If the party seeking summary judgment meets this burden, then summary judgment will be granted unless there is significant probative evidence tending to support the opponent's legal theory. *First National Bank of Arizona v. Cities Service Co.*, 391 U.S. 253, 290, 88 S. Ct. 1575, 1593, 20 L. Ed. 2d 569 (1968); *Commodity Futures Trading Commission v. Savage*, 611 F.2d 270 (9th Cir. 1979). Parties seeking to defeat summary judgment cannot stand on their pleadings once the movant has submitted affidavits or other similar materials. Affidavits that do not affirmatively demonstrate personal knowledge are insufficient. *British Airways Bd. v. Boeing Co.*, 585 F.2d 946, 952 (9th Cir. 1978), *cert. denied*, 440 U.S. 981, 60 L. Ed. 2d 241, 99 S. Ct. 1790 (1979). Likewise, "legal memoranda and oral argument are not evidence and do not create issues of fact capable of defeating an otherwise valid motion for summary judgment." *Id.*

A material issue of fact is one that affects the outcome of the litigation and requires a trial to resolve the differing versions of the truth. *See Admiralty Fund v. Hugh Johnson & Co.*, 677 F.2d 1301, 1305-06 (9th Cir. 1982); *Admiralty Fund v. Jones*, 677 F.2d 1289, 1293 (9th Cir. 1982).

All facts and inferences drawn must be viewed in the light most favorable to the responding party when

determining whether a genuine issue of material fact exists for summary judgment purposes. *Poller v. CBS*, Inc., 368 U.S. 464, 7 L. Ed. 2d 458, 82 S. Ct. 486 (1962). After drawing inferences favorable to the respondent, summary judgment will be granted only if all reasonable inferences defeat the respondent's claims. *SEC v. Seaboard Corp.*, 677 F.2d 1297, 1298 (9th Cir. 1982).

The trilogy of Supreme Court cases cited above establishes that "summary judgment procedure is properly regarded not as a disfavored procedural shortcut, but rather as an integral part of the Federal Rules as a whole, which are designed 'to secure the just, speedy and inexpensive determination of every action.'" *Celotex Corp.*, 477 U.S. at 327, quoting Fed. R. Civ. P. 1). *See also Avia Group Int'l, Inc. v. L.A. Gear Cal.*, 853 F.2d 1557, 1560 (Fed. Cir. 1988).

II. ERISA

A. Facts & Procedural History

In their First Claim for Relief, Plaintiffs' Co-Payor Class seeks approximately $ 85 million in damages against Defendant Humana Insurance for violation of its fiduciary duties under ERISA.

At the heart of this entire case is a provision within the Humana Care Plus Policy under which members of the Co-Payor Class were insured. This provision states that the employee insured would pay 20% of all covered charges incurred while in a hospital up to a certain amount (usually $ 5,000), and that Humana Insurance would be responsible for the remaining 80%. See Opposition (#820), Exhibit 85

at 7-1 (Humana Insurance master group health insurance policy HCP-84-NV-1).[1] From 1984 to 1988, Humana Insurance obtained substantial discounts, allegedly between 40% and 96%, from Sunrise for services rendered to the insureds in the Co-Payor Class. Errata (#822) at 49-50, Exhibit #39; Opposition (#820), Exhibits 38, 40. Instead of sharing these discounts with the insureds by reducing the amount of co-payments due, Humana Insurance only applied the discount to the amount for which they would have been responsible under the policy. Motion for Summary Judgment (#804) at 10-12.

Based on these actions, the Co-Payor Class asserts that Humana Insurance breached several of its fiduciary duties under ERISA including:

(1) Humana Insurance engaged in a transaction involving the plan on behalf of a party, Sunrise, whose interests were adverse to those of the health benefits plans and/or its participants or beneficiaries in violation of 29 U.S.C. § 1106(b)(2);

(2) Humana Insurance dealt with plan assets in its own interest or for its own account by erroneously interpreting the individual insurance contracts as requiring co-payments based on gross charges and effectuating the co-payment

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[1] Accordingly, in addition to any premiums which were paid to Humana Insurance by the employer or employee, the employee insured would ultimately be responsible for up to $ 1,000 (20% of $ 5,000) before Humana would absorb 100% of covered hospital charges. Errata (#822) at 123-125.

billings through contract administration in violation of 29 U.S.C. § 1106(b)(1);

(3) Humana Insurance received consideration (e.g. the discounts) from Sunrise involving the assets of the plan in violation of 29 U.S.C. § 1106(b)(3);

(4) Humana Insurance did not act in the sole interest of the insureds in interpreting the insurance contracts in violation of 29 U.S.C. § 1104(1)(A)(i);

(5) Humana Insurance failed to discharge its duties with respect to the plan in accordance with the documents and instruments governing the plan in violation of 29 U.S.C. § 1104(1)(D).

See Errata (#822) at 173-193.

On September 26, 1989, this Court denied Defendant Humana Insurance's Motion to Dismiss (#13) the Co-Payor Class' claim for breach of fiduciary duty. In denying Humana Insurance's Motion, this Court held, citing *Sixty-Five Security Plan v. Blue Cross and Blue Shield*, 583 F. Supp. 380, 387 (S.D.N.Y. 1984), *aff'd on reh'g*, 588 F. Supp. 119 (1984), that given the discretion of Humana Insurance to negotiate reimbursement rates on behalf of the health benefit plans, Humana Insurance should be considered a fiduciary under ERISA. Order (#79) at 9. This Court further held that the plain meaning of the contract language indicated that the insureds should have paid 20% of the net hospital charge and not the gross charge, and consequently, by failing to distribute the negotiated discount among the Co-Payor Class, Humana Insurance may have violated its fiduciary duties. *Id.* at 10. Finally, this Court found that

Humana Insurance's failure to reduce its insurance premiums did not constitute a breach of fiduciary duty and that the establishment of co-payment rates was distinguishable from the fiduciary duties imposed in the approval or denial of insurance claims. *Id.* at 6-9, 10.

On July 9, 1991, this Court entered a second Order (#364) which again refused to dismiss the Co-Payor Class' claim for breach of fiduciary duties under ERISA. In the Order, this Court reaffirmed its holding that Humana Insurance was a fiduciary under *Sixty-Five Security Plan* and acknowledged that the Co-Payor Class was also asserting that Humana Insurance engaged in prohibited transactions in violation of 29 U.S.C. § 1106(b). Order (#364) at 4. This Court went on to hold that Plaintiffs were not asserting a claim for benefits[2], but instead were "seeking recovery for having overpaid hospital stays, because Humana Insurance as fiduciary should have acted in their interest and assured that Plaintiffs were not paying based on a 'fraudulent' rate." *Id.* at 6. In addition, in rejecting Humana Insurance's argument that Plaintiffs could not assert a claim for breach of fiduciary relief in their individual capacity, this Court found that:

> This case presents a unique situation in that the whole plan was affected by the alleged breach by Humana Insurance, but there was only monetary harm to individual participants and beneficiaries if

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[2] In the Order, this Court defined a "claim for benefits" as a situation where "an insured puts in a claim and the insurance company denies it." Order (#364) at 6.

and when they were required to pay copayments. The only meaningful remedy is recovery by these individual Plaintiffs, and the provisions of § 1132(a)(3) permit such recovery. *Id.* at 14.[3]

In support of the instant Motion for Summary Judgment on Plaintiffs' ERISA claim, Defendant Humana Insurance advances three alternative arguments: (1) Plaintiffs are precluded from bringing a claim for breach of fiduciary duty since they are seeking relief to benefit only themselves (e.g., the Co-Payor Class) and not the plan as a whole[4]; (2)

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[3] In so doing this Court primarily relied upon a broad interpretation of *Amalgamated Clothing & Textile Workers Union v. Murdock*, 861 F.2d 1406 (9th Cir. 1988) and distinguished the holdings of *Massachusetts Mutual Life Ins. Co. v. Russell*, 473 U.S. 134, 87 L. Ed. 2d 96, 105 S. Ct. 3085 (1985) and *Sokol v. Bernstein*, 803 F.2d 532 (9th Cir. 1986).

[4] Defendant Humana Insurance claims that the total number of insureds who had a co-payment obligation to Humana Hospital Sunrise as a percentage of the number of total insureds was 11.68% (16,412/140,487), whereas the total number of insureds whose co-payment obligation would have declined if their coinsurance was calculated on the discounted amount as a percentage of the number of total insureds was 9.54% (13,049/140,487). Plaintiffs state that the 9.54% is not supported by the evidence, but do concede that not all employees covered under the various health plans were hospitalized at Humana Hospital Sunrise. Opposition (#820), Exhibit 93; Errata (#822) at 207, n.404.

Humana Insurance did not act as a fiduciary in entering into the discount arrangement with Sunrise; and (3) if a claim for breach of fiduciary duty is allowed, monetary relief should be limited to recovery of the allegedly excessive coinsurance payments. Motion for Summary Judgment (#804) at 14-35.

Plaintiffs respond that: (1) this Court has already correctly ruled that regardless of the fact that only the Co-Payor Class was financially injured by Humana Insurance's discounting scheme, since all the insureds were "affected" by the scheme, the Co-Payor Class may properly pursue a constructive trust for Humana Insurance's alleged violations of fiduciary duty; (2) this Court correctly ruled that Humana Insurance was rendered a fiduciary by negotiating a discount with Humana Hospital Sunrise since such an act constituted discretion in the administration of a plan; and (3) under ERISA, damages for Humana Insurance's alleged breach of fiduciary duty should be measured by the sum of the total discount received by Humana Insurance from Sunrise and not simply the percentage of the discount that Humana Insurance should have shared under the terms of the insurance contract.

B. Discussion

1. May the Co-Payor Class Assert a Claim for Breach of Fiduciary Duty?

Notwithstanding this Court's Order (#364) of July 9, 1991, Humana Insurance once again asserts that the Co-Payor Class may not pursue its claims for breach of fiduciary duty, as the class seeks monetary relief on behalf of themselves as individuals and not on behalf of their

respective employee benefit plans. In support of its position, Humana Insurance cites two recent Ninth circuit cases, *Horan v. Kaiser Steel Retirement Plan*, 947 F.2d 1412 (9th Cir. 1991) and *Williams v. Caterpillar, Inc.*, 944 F.2d 658 (9th Cir 1991), as new authority upon which this Court should rely to reverse its earlier decision. Although it does not appear that either of these cases charts new waters in the rough seas of ERISA interpretation, these decisions do present a forceful argument for reconsidering this Court's earlier interpretation of *Russell*, *Sokol*, and *Murdock*. See Order (#364) at 7-14.

To begin, this Court reaffirms its earlier interpretation of *Russell* to the effect that the substantive remedies afforded by 29 U.S.C. § 1109 are limited, and that § 1109 does not allow for the recovery of extra-contractual damages by individual beneficiaries. See Order (#364) at 8-9; *Russell*, 473 U.S. at 142-144. In coming to this conclusion, the Supreme Court in *Russell* specifically found that even though ERISA requires plan fiduciaries to serve the interests of participants and beneficiaries, § 1109 provides remedies for breach of fiduciary duties only on behalf of the plan, and not the individual beneficiary.[5] *Id.* Subsequently, the Ninth

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[5] More specifically, the Supreme Court stated:

> A fair contextual reading of (@ 1109) makes it abundantly clear that its draftsmen were primarily concerned with the possible misuse of plan assets, and with remedies that would protect the entire plan, rather than with the rights of an individual beneficiary. . . . *[T]he entire text of (@ 1109) persuades us that Congress did not intend to authorize any relief except for the plan itself.*

Russell, 473 U.S. at 142, 144 (emphasis added).

Circuit adopted the reasoning used in *Russell* to foreclose the availability of extra-contractual damages under 29 U.S.C. § 1132(a)(3). *See Sokol*, 803 F.2d at 534-537. In doing so, the Ninth Circuit emphasized, just as the Supreme Court did in *Russell*, that the remedies allowed under § 1132(a)(3) are limited and that § 1132(a)(3) "emphasizes the protection of the plan, not the direct protection of the beneficiaries." *Id.* at 536-537.[6]

The two recent cases cited by Defendant Humana Insurance, *Horan* and *Williams*, restate the interpretation of *Russell* and *Sokol* summarized above. For example, in holding that Plaintiffs in Horan could not bring a claim for breach of fiduciary duty under § 1132(a)(3), the Ninth Circuit stated:

> An individual beneficiary may bring a fiduciary breach claim, but must do so for the benefit of the plan. An individual beneficiary may not pursue a fiduciary breach claim to recover benefits or remedies beyond those provided by a plan. Any

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[6] This Court also recognizes that the position adopted by the Ninth Circuit in Sokol directly contradicts Justice William Brennan's interpretation of @ 1132(a)(3) found in his concurring opinion in *Russell*. See Order (#364) at 9; *Russell* (Brennan, J., concurring) at 153-155; *Sokol*, 803 F.2d at 534-538.

recovery for a violation of sections 1109 and 1132(a)(2) must be on behalf of the plan as a whole, rather than inuring to individual beneficiaries. The Supreme Court reasoned the fiduciary duty provisions in ERISA are primarily concerned with protecting the integrity of the plan, which in turn protects all beneficiaries, rather than remedying each wrong suffered by individual beneficiaries.

We have extended the reasoning in *Russell* to section 1132(a)(3) which allows recovery of 'other appropriate equitable relief.' Section 1132(a)(3) also does not provide an action for an individual beneficiary to recover extra-contractual remedies. Under *Russell* and *Sokol*, the plaintiffs fail to present a fiduciary breach claim if the only remedy sought is for their own benefit, rather than for the benefit of the Plan as a whole.

Horan 947 F.2d at 1417-18 (citations omitted). Similarly, in *Williams* the Ninth Circuit recognized that "the Supreme Court has held that equitable relief under ERISA is limited to relief protecting the integrity of the plan as a whole and does not extend to individual plan participants." *Williams*, 944 F.2d at 665.

In this Court's previous Order (#364) of July 9, 1991, it found that, based on *Murdock* and § 1132(a)(3), the only "meaningful remedy" in this case would be to allow the Co-Payor Class to sue Humana Insurance for breach of fiduciary duty. See Order (#364) at 14. In light of the additional clarification yielded by the *Horan* and *Williams*,

this Court now recognizes that this conclusion was incorrect.

First, it is undisputed that the members of the Co-Payor Class are seeking approximately $ 85 million in damages in their individual capacity for various breaches of fiduciary duty pursuant to § 1132(a)(3). As set forth in *Sokol*, *Horan*, and *Williams*, however, § 1132(a)(3) does not allow individual beneficiaries to sue for a breach of fiduciary duty on their own behalf. *See Horan* 947 F.2d at 1417-18; *Williams*, 944 F.2d at 665; *Sokol*, 803 F.2d at 536-537.

Second, this Court's previous reliance on *Murdock* as authority for allowing the Co-Payor Class to seek damages under § 1132(a)(3) must be acknowledged as tenuous. Although the Court in *Murdock* did allow plan beneficiaries (and not the plan itself) to benefit from a constructive trust funded with the ill-gotten profits of the plan administrator, the Court in *Murdock* also limited its ruling to the "circumstances of this case." *Murdock*, 861 F.2d at 1417. These circumstances included the fact that (1) the ERISA plan which had been defrauded no longer existed, (2) under the terms of the plan (which were altered by the errant fiduciary) any damages paid to the plan would have simply reverted to the wrongdoer, and (3) the damages to be received for the breach of fiduciary duty were to be put into a constructive trust in favor of all plan participants. *Id.* at 1408-09, 1417.

Here, although Defendants' conduct as alleged by Plaintiffs is clearly questionable, the level of impropriety

alleged is less severe than that which occurred in *Murdock*.[7] The injury in *Murdock* was directed primarily against the plan and not against particular beneficiaries, the various benefit plans purportedly affected by Humana Insurance's breach of fiduciary duty still exist, and, most importantly, this Court's prior Order (#364) would afford relief only to a select group of beneficiaries. Upon reconsideration and based on these distinctions, this Court finds that *Murdock* does not provide persuasive authority to support this Court's earlier interpretation of § 1132(a)(3).[8]

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[7] In *Murdock*, the fiduciary used plan assets to further a "greenmail" scheme to benefit his own individual assets. After the scheme was successful (with both the plan and the fiduciary making substantial profits as a result), the fiduciary then amended the plan so that any excess funds would revert to the administrator. With the plan so changed, the fiduciary then paid all beneficiaries the value of their vested interests under the plan and retained all excess monies for himself (e.g. the profits made by the plan based on the use of its assets in the greenmail scheme). *Murdock*, 861 F.2d at 1408-1409.

[8] In the July 9, 1991 Order (#364), this Court also cited *Call v. Sumitomo Bank*, 881 F.2d 626, 631-32 (9th Cir. 1989) for the proposition that the *Russell* decision should not be narrowly construed. This Court now concludes that the language used for this proposition was dicta, and furthermore, that the Court in *Call* itself recognized that "'ERISA grants no private right of action by a beneficiary *qua* beneficiary; rather it *accords beneficiaries the right to sue on behalf of the entire plan* if a fiduciary breaches the plan's terms.'" *Id.* at 632, n. 13 (citing *Sokol*). *See also*

Finally, this court also recognizes that a claim for breach of fiduciary duty is neither the only nor the most appropriate remedy for the wrongs asserted by the Co-Payor Class. After careful consideration of the facts and the injury alleged by the Co-Payor Class in this case, it is clear that the crux of the current dispute rests on the reasonableness of Humana Insurance's interpretation of the co-payment provision in the insurance contract. Such a claim is more appropriately brought pursuant to 29 U.S.C. § 1132(a)(1)(B) which allows a beneficiary to seek benefits due under the terms of the plan. *See e.g., Eaton v. Blue Cross and Blue Shield of Alabama*, 681 F. Supp. 759, 760-62 (S.D. Ala. 1988). Accordingly, this Court finds that the cause of action provided by ERISA to compensate the Co-Payor Class for the questionable conduct of Humana Insurance is a claim for benefits pursuant to § 1132(a)(1)(B) and not a claim for breach of fiduciary duty under §1109 or § 1132(a)(3).[9]

Murdock, 861 F.2d at 1417.

[9] This Court also recognizes that in its Motion for Summary Judgment (#804), Humana Insurance concedes that such a claim is proper, and has stipulated that to the extent the Co-Payor Class would be required to exhaust their administrative remedies prior to bringing a claim for benefits, such a requirement would be waived. See Motion for Summary Judgment (#804) at 22. In addition, this Court finds that to avoid any unjust enrichment on the part of Humana Insurance, Plaintiffs would be entitled individually to obtain prejudgment interest for any relief provided under @ 1132(a)(1)(B). *Rivera v. Benefit Trust Life Ins. Co.*, 921 F.2d 692 (7th Cir. 1991); *Short v.*

In sum, based on the holdings in *Horan*, *Williams* and *Sokol*, the limited precedential value of *Murdock*, and the availability of a more appropriate remedy pursuant to § 1132(a)(1)(B), this Court finds that the Co-Payor Class should not be allowed to pursue claims against Humana Insurance for breach of fiduciary duty. Accordingly, this Court hereby modifies its earlier Order (#364) of July 8, 1991, and will grant Humana Insurance's Motion for Summary Judgment as to the first claim for relief set forth in Plaintiffs' Second Amended Complaint (#370). Plaintiffs' Co-Payor Class will be permitted, however, to file a Third Amended Complaint seeking appropriate relief under § 1132(a)(1)(B).

2. Damages

This Court also finds that even if it were to allow Plaintiffs' Co-Payor Class to seek a claim for breach of fiduciary duty, Humana Insurance would still be entitled to partial summary judgment on the issue of damages.

Plaintiffs argue that "equity and poetic justice" require that Humana Insurance pay to the beneficiaries the entire discount it obtained from Sunrise.[10] Errata (#822) at 126. In

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Central States, S.E. & S.W. Areas Pension Fund, 729 F.2d 567, 576 (8th Cir. 1984); *see also Blanton v. Anzalone*, 760 F.2d 989, 992-93 (9th Cir. 1985).

[10] The following discussion regarding the proper manner in which to calculate damages is applicable to a theory of liability predicated upon both @ 1132(a)(1)(B) and breach of fiduciary duty. Thus, although this Court has

contrast, Humana Insurance asserts that Plaintiffs' calculations are incorrect, and that the appropriate measure of damages in this case would be simply a refund of the overcharged co-payment. See Reply (#841) at 22-27.

Section 1109(a) provides that:

> Any person who is a fiduciary with respect to the plan who breaches any of the responsibilities, obligations, or duties imposed upon fiduciaries by this subchapter shall be personally liable to make

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disallowed Plaintiffs' fiduciary duty claim, this ruling does not affect the measure of damages to be awarded to Plaintiffs. Using the example provided by Plaintiffs, assume the following:

(1) Gross hospital charge = $ 5,000.
(2) Amount paid by insured = $ 1,000 (.20 X $ 5,000).
(3) Amount paid by Humana Insurance after taking an 89% discount = $ 550 (.89 X $ 5,000).
(4) Total amount paid to Sunrise = $ 1,550 ($ 1,000 + $ 550).

Plaintiffs argue that appropriate damages for Humana Insurance's breach of fiduciary duty would be to require it to "disgorge the entire 'secret' discount" obtained by Humana Insurance from Sunrise. Based on their statement that "disgorgement of the unilateral discounts actually requires Humana Insurance to pay 80% of the gross charges," this amount would presumably be $ 3,450 ($ 4,000 (.80 X $ 5,000) - $ 550). See Errata (#822) at 123-26.

> good to such plan any losses to the plan resulting from such breach, and to restore to such plan any profits of such fiduciary which have been made through use of assets of the plan by the fiduciary, and shall be subject to such other equitable or remedial relief as the court may deem appropriate, including the removal of such fiduciary.

Similarly, under § 1132(a)(3), a "beneficiary" may seek "other appropriate equitable relief" to redress breaches of fiduciary duty or to enforce the provisions or terms of the plan.

"Where there has been a breach of fiduciary duty, ERISA grants to the Courts broad authority to fashion remedies for redressing the interests of participants and beneficiaries." *Donovan v. Mazzola*, 716 F.2d 1226, 1235 (9th Cir. 1983), *cert. denied*, 464 U.S. 1040, 79 L. Ed. 2d 169, 104 S. Ct. 704 (1984). Notwithstanding this, any loss or profit which is disgorged as a result of a breach of fiduciary duty must have a causal connection with the breach. *Leigh v. Engle*, 727 F.2d 113, 137 (7th Cir. 1984). Uncertainties in the determination of damages, however, should be resolved against the wrongdoer. *Kim v. Fujikawa*, 871 F.2d 1427, 1431 (9th Cir. 1989); *Leigh*, 727 F.2d at 138-39.

Plaintiffs argue that they are entitled to the entire discount yielded by Sunrise as this amount constitutes the ill-gotten profits Humana Insurance obtained as a result of its breach. The difficulty with Plaintiffs' position, however, is that their calculation ignores the facts of the case and has no "causal connection" to the breach.

To illustrate this, assume, as Plaintiffs do, that on a gross hospital charge of $ 5,000, the insured beneficiary paid $ 1,000 in co-payments and Humana Insurance paid $ 550 on its obligation. The net amount received by Sunrise equals $ 1,550, and the net discount given by the hospital amounts to $ 3,450.[11] Assuming Plaintiffs are correct in their interpretation of the co-payment provision that the insureds were responsible for 20% of the net charges and not the gross charges,[12] the amount which should have been paid by the insureds is $ 310 (.20 X $ 1,550). Therefore, since the insured paid $ 1,000 rather than the $ 310 which was required under the contract, the insured overpaid, and Humana Insurance profited, $ 690.

As illustrated above, it is clear that the amount asserted by Plaintiffs, or $ 3,450, is not a true measure of Humana Insurance's "ill-gotten profit." To adopt this figure would ignore the reality that the gross hospital charge ($ 5,000) was never in fact paid and would contradict Plaintiffs' interpretation of the co-pay provision that the 20/80 split should apply only on the net hospital charge ($1,550). Accordingly, since Plaintiffs' measure of damages fails to

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[11] Note that the net discount given on the hospital bill is 69% ($ 3,450/$ 5,000) and the net discount given on the portion of the bill for which Humana Insurance was responsible is 86.25% ($ 3,450/$ 4,000) Accordingly, Plaintiffs' assertion that Humana Insurance received an 89% discount is inaccurate. See *supra*, n. 10; Errata (#822) at 124-125.

[12] See Second Amended Complaint (#370) at 8.

establish a "causal connection" between the actual profits obtained by Humana Insurance and its breach of fiduciary duty, their calculation of damages must be rejected.

The proper measure of damages would be to restore the trust beneficiaries to the position they would have occupied but for the breach of trust. *Donovan v. Bierwirth*, 754 F.2d 1049, 1056 (2nd Cir. 1985). Applying this remedy to the facts of the present situation, Humana Insurance would be required to disgorge the amount of the discount which it should have paid under the proper interpretation of the co-pay provision. Using the hypothetical insured described by Plaintiffs, this would amount to $ 690. In so doing, the primary purpose of § 1109(a) to "undo" the harm caused by the breach of fiduciary duty would be realized. The interests of the beneficiaries would be furthered by removing the fiduciary's ill-gotten profits, and consistent with the holding in *Leigh*, only those profits which are "causally connected" with Humana Insurance's breach of fiduciary duty would be disgorged. *See Leigh*, 727 F.2d at 138-139; *Bierwirth*, 754 F.2d at 1056.[13]

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[13] In their Fourth Supplemental Opposition (#896) Plaintiffs argue that the recent United States Supreme Court case *Mertens v. Hewitt Assocs.*, 124 L. Ed. 2d 161, 113 S. Ct. 2063, 61 U.S.L.W. 4510 (1993), stands for the proposition that this Court has the ability to create a trust composed of all discounts yielded by Sunrise to Humana Insurance as a remedy for Humana Insurance's alleged breach of fiduciary duty. This Court disagrees. The primary holding of *Mertens* is that "appropriate equitable relief" under @ 1132(a)(3) does not permit compensatory

Accordingly, even if the Co-Payor Class could assert a claim for breach of fiduciary duty under the present facts, partial summary judgment would still be appropriate in this case with respect to Plaintiffs' excessively broad view of "ill-gotten profits."[14]

Leave to Amend Plaintiffs' Complaint.

In their Opposition, Plaintiffs argue that in the "unlikely event that this Court completely reverses itself and rules that ERISA offers no remedy to Plaintiffs," this Court should allow Plaintiffs to reinstate their state law claims. Errata (#822) at 95, n.2. This Court disagrees.

First, under this Order, Plaintiffs are not without an ERISA remedy. The Co-Payor Class properly may file an Amended Complaint to state a claim under § 1132(a)(1)(B). See *supra* at 17. Moreover, Plaintiffs' state law claims (which would apply to Plaintiffs' recent proposed claim

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losses for breaches of fiduciary duty, but rather only traditional forms of equitable relief such as injunction, mandamus, and restitution of ill-gotten profits. 61 U.S.L.W. at 4513-14. Such a holding does not aid Plaintiffs because, as discussed above, Plaintiffs' argument that the entire discount yielded by Sunrise to Humana Insurance are Humana Insurance's "ill-gotten profits" is erroneous.

[14] In light of this Court's findings regarding the Co-Payor Class' inability to pursue a claim for breach of fiduciary duty, this Court need not address Defendants' second argument that Humana Insurance was not a fiduciary when it negotiated the discount with Sunrise.

under Nevada's RICO statute, N.R.S. § 207.470) are preempted by ERISA. See Order (#79) at 14-15; *Olson v. General Dynamics Corp.*, 960 F.2d 1418, 1420-23 (9th Cir. 1991), *cert. denied*, 119 L. Ed. 2d 588, 112 S. Ct 2968 (1992); *Kanne v. Connecticut General Life Ins. Co.*, 867 F.2d 489, 493-494 (9th Cir. 1988), *cert. denied*, 492 U.S. 906, 106 L. Ed. 2d 566, 109 S. Ct. 3216 (1980).

Finally, as this Court has indicated previously, given the procedural history of this case which has seen three extensive rounds of dispositive motions, to allow the parties to address new theories of liability would strain the letter and spirit of Fed. R. Civ. P. 15(a).

Accordingly, Plaintiffs' request to reinstate their prior state law claims was properly denied in this Court's October 13, 1992 Order (#840), and will not be reconsidered here.

III. ANTITRUST

A. Facts

Plaintiffs' Second Claim for Relief seeks approximately $ 181 million in damages against Defendants Humana and Humana Insurance for monopolization or attempted monopolization under section 2 of the Sherman Act, 15 U.S.C. § 2. The "Antitrust Class" consists of individual employees and employers who allegedly paid excessive policy premiums and co-payments to Humana and Humana Insurance between 1985 and 1988. See Second Amended Complaint (#370) at 11, P 41; 17, P 51.

In their opposition, Plaintiffs argue that Humana and Humana Insurance monopolized or attempted to monopolize two independent submarkets including: (1) Clark County for-profit acute care hospitals; and (2) those hospitals used by Humana Insurance's insureds. Errata (#822) at 290-292.

As proof of this actual or attempted monopolization, the Antitrust Class alleges that during the relevant time period, Humana made "supracompetitive" or "monopoly" profits.[15] Plaintiffs maintain that with regard to the for-profit acute care hospital submarket, Sunrise had "60% of the licensed beds" and derived "58.6% of the total gross patient revenues excluding medicare and medicaid patients" during the relevant time period. *Id.* at 305. Also, with respect to the submarket of hospitals used by Humana Insurance's

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[15] In support of the assertion that Humana made "supracompetitive" profits, Plaintiffs allege that: (1) Nevada hospitals have the highest profit margins in the country and that Sunrise was the most profitable hospital in Nevada; (2) Sunrise was the most profitable hospital in the Humana organization; (3) Notwithstanding the fact that Sunrise's market share decreased during the relevant time period, its net operating revenue during this period increased; and (4) During 1988 when Nevada's cost containment statute was in place, Sunrise's operating margin was excessive compared to other hospitals nationally. Errata (#822) at 278-81.

insureds, Plaintiffs allege that Sunrise enjoyed an 80% share within this market. *Id.*[16]

Plaintiffs claim that in order to maintain or enhance its monopolistic position, Defendants acted anti-competitively in several ways, including: (1) Humana refused to unbundle level III neonatal care to other health care plans; (2) Humana improperly diverted critical care patients to other hospitals; (3) Humana threatened to revoke physician office space if they did not admit patients to Sunrise; (4) the discount arrangement between Humana and Humana Insurance allowed Sunrise to understate hospital profits and avoid regulatory cost containment while misleading its insureds; (5) Humana threatened physicians and lawmakers who challenged its practices; and (6) Humana Insurance improperly tied use of its insurance policy with treatment at Sunrise. Errata (#822) at 283-290.

Finally, with regard to the antitrust injury suffered by the Antitrust Class, Plaintiffs allege that, as a result of Defendants' monopolistic activity, its members were forced to pay excessive co-payments and insurance premiums. Errata (#822) at 276-77, 308-311.

B. Discussion

1. Elements of a Section 2 Claim

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[16] The only evidence that Plaintiffs submit in support of this allegation of market share is a December 7, 1984 memo which states that "non-Humana usage was 19%" for Humana insurance insureds. Errata (#822) at 305; Exhibit 132.

In order to prevail on a claim of monopolization under Section 2 of the Sherman Act, a plaintiff must demonstrate: (1) possession of monopoly power in the relevant market; (2) willful acquisition or maintenance of that power; and (3) causal antitrust injury. *Oahu Gas Service, Inc. v. Pacific Resources, Inc.*, 838 F.2d 360, 363 (9th Cir. 1988), *cert. denied*, 488 U.S. 870, 102 L. Ed. 2d 149, 109 S. Ct. 180 (1988).

To establish a Section 2 claim for an attempt to monopolize, a plaintiff must demonstrate four elements: (1) specific intent to control prices or destroy competition; (2) predatory or anti-competitive conduct directed toward accomplishing that purpose; (3) a dangerous probability of success; and (4) causal antitrust injury. *McGlinchy v. Shell Chemical Co.*, 845 F.2d 802, 811 (9th Cir. 1988).

Both market definition and market power are essentially questions of fact, whereas questions of whether specific conduct is anti-competitive or whether a party has standing to assert a violation of the Sherman Act are questions of law. *Austin v. Blue Cross and Blue Shield of Alabama*, 903 F.2d 1385, 1387 (11th Cir. 1990); *Oahu*, 838 F.2d at 363, 368.

2. The Need for Market Analysis

As an initial matter, Plaintiffs argue that they need not formally define a "relevant market," as Humana and Humana Insurance made "supracompetitive profits" during the relevant time period, and that this alone adequately demonstrates market power. Errata (#822) at 278-283. Such an argument is without merit.

First, by definition, any allegation that Defendants made "excessive" profits necessarily requires that Plaintiffs compare the profit performance of Humana with that of its competitors. See Kinter, *Federal Antitrust Law*, § 12.10, at 361 (1980). To argue, as Plaintiffs do, that Defendants' profits were "excessive" without knowing what the relevant market was or who its competitors were renders any antitrust analysis vacuous.

Second, although prior Ninth Circuit law has held that a detailed market analysis is not uniformly fatal to a claim under the Sherman Act, in order to avoid such an analysis a plaintiff must be able to demonstrate that the anti-competitive acts alleged actually resulted in detrimental effects on competition. *Bhan v. NME Hosp., Inc.*, 929 F.2d 1404, 1413 (9th Cir.), *cert. denied*, 116 L. Ed. 2d 639, 112 S. Ct. 617 (1991); Morgan, Strand, Wheeler & Biggs v. Radiology, Ltd.24, 924 F.2d 1484, 1489 n.3 (9th Cir. 1991); *Thurman Industries, Inc. v. Pay 'N Pak Stores, Inc.*, 875 F.2d 1369, 1373 (9th Cir. 1989); *Oltz v. St. Peter's Community Hospital*, 861 F.2d 1440, 1448 (9th Cir. 1988) (no need for detailed market analysis where evidence clearly demonstrated competitive harm to patient market in Helena through increase of price and decrease in competition for anesthesiological services). In the present case, Plaintiffs rely on conclusory allegations that Defendants made excessive profits and do not explain how these profits have resulted from anti-competitive conduct. See Errata (#822) at 278-83. Such allegations clearly do not demonstrate a definable

market or demonstrate market power which would obviate the need of a proper market analysis.[17]

Finally, even though courts have allowed proof of excessive profits to be used as evidence of market power, such proof may be misleading and subject to various interpretations. See Kinter, § 12.10; Areeda & Turner, *Antitrust Law*, § 507-516 (1978). As such, citing excessive profits without a more detailed analysis of the meaning of such profits is unsatisfactory proof of market power. *Telerate Systems, Inc. v. Caro*, 689 F. Supp. 221, 238 (S.D.N.Y. 1988).

Accordingly, since allegations of Defendants' "excessive" profits do not clearly demonstrate market power or detrimental effects on competition, Plaintiffs still have the burden of proving that Defendants exercised monopoly power within the boundaries of a properly defined competitive market.

3. Defining the Relevant Market

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[17] This Court also recognizes that the Ninth Circuit's decisions on this point have been called into doubt by the United States Supreme Court's recent decision, *Spectrum Sports, Inc. v. McQuillan*, 122 L. Ed. 2d 247, 113 S. Ct. 884 (1993). *Spectrum Sports* held that an inquiry into market definition and market power is a prerequisite for a claim of attempted monopolization under @ 2 of the Sherman Act. *Id.* at 891-92. Even without regard to *Spectrum Sports*, however, it is clear that Plaintiffs have not satisfied Ninth Circuit precedent which limits the necessity of a market analysis.

Under Section 2, "defining the relevant market is indispensable to a monopolization claim." *Thurman Industries, Inc.*, 875 F.2d at 1373. A market is typically defined as the pool of goods or services that qualify as economic substitutes because they enjoy reasonable interchangeability of use and cross-elasticity of demand. *Morgan*, 924 F.2d at 1489; *Thurman*, 875 F.2d at 1373. The determination of a market usually requires an inquiry into the nature of the product and the geographic areas of effective competition. *Oahu*, 838 F.2d at 364. "For antitrust purposes, defining the product market involves the identification of the field of competition; the group or groups of sellers or producers who have actual or potential ability to deprive each other of significant levels of business," while "a geographic market is an area of effective competition where buyers can turn for alternate sources of supply." *Morgan*, 924 F.2d at 1489-90; *see also Bhan*, 669 F. Supp. 998, 1018-19 (E.D.Cal. 1987), *affirmed*, 929 F.2d 1404 (9th Cir.), *cert. denied*, 116 L. Ed. 2d 639, 112 S. Ct. 617 (1991) (another consideration in defining product market may be production dimension which is the ability of competing firms to easily supply a good or service if one firm limits its output). Finally, the plaintiff has the burden of proving the scope of the relevant market. *Id.* at 1019.

4. Plaintiffs' Relevant Markets

Plaintiffs offer two distinct markets which they contend Defendants monopolized or attempted to monopolize. These include: (1) those hospitals used by Humana Insurance insureds; and (2) all major for-profit acute care hospitals in Clark County, Nevada. Errata (#822) at 290-292. Plaintiffs,

however, fail to present sufficient evidence to support these market definitions.

a. Hospitals Used by Humana Insurance Insureds

With regard to those hospitals used by Humana Insurance insureds, Plaintiffs offer no facts to support such a market definition other than to refer to a recent United States Supreme Court decision *Eastman Kodak Co. v. Image Technical Services*, 119 L. Ed. 2d 265, 112 S. Ct. 2072 (1992), and conclude that the hospitals used by Humana Insurance insureds should be considered a relevant submarket. Errata (#822) at 291-292; 304-305. Providing no factual or legal basis for such a submarket, Defendants are entitled to summary judgment on this aspect of Plaintiffs' Section 2 claims. See *Seidenstein v. National Medical Enterprises, Inc.*, 769 F.2d 1100, 1106 (5th Cir. 1985); *Drs. Steuer & Latham v. Nat. Med. Enterprises*, 672 F. Supp. 1489, 1514 (D.S.C. 1987).[18]

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[18] Even if this Court were to accept the argument that the hospitals used by Humana Insurance insureds could constitute a relevant submarket, Plaintiffs fail to address: (1) the fact that Humana insureds were able to (and did) easily switch health insurance policies; (2) the fact that under the Humana policy, Plaintiffs were not precluded from choosing hospitals other than Sunrise; and (3) what other hospitals were competing within this market. See Reply (#841) at 72-75. Failure to address these facts clearly precludes finding the existence of Plaintiffs' proposed submarket.

b. Major For-Profit Acute Care Hospitals

The second market which Plaintiffs maintain Defendants monopolized or attempted to monopolize is that of major for-profit acute care hospitals in Clark County. This market allegedly consists of three hospitals, including Sunrise, valley Hospital ("Valley"), and Desert Springs Hospital ("Desert Springs"), and excludes Clark County's five remaining acute care hospitals, Boulder City Hospital ("Boulder City"), Community Hospital of North Las Vegas ("Community"), St. Rose De Lima Hospital ("St. Rose"), University Medical Center ("UMC"), and Women's Hospital ("Women's"). Errata (#822) at 291. In support of this market definition, Plaintiffs assert the following facts:

(1) Only Sunrise, Valley, Desert Springs, and University Medical Center ("UMC") have a comparable range of hospital services;

Further, to the extent Plaintiffs rely on the *Kodak* decision to support a claim for an illegal tying arrangement between Humana and Humana Insurance, such a claim is without merit as the undisputed facts indicate that Humana Insurance never possessed sufficient market share to exercise market power within the group health insurance market. See Reply (#841), Exhibit N, Supplemental Affidavit of Barry C. Harris at 16-22 (Humana Insurance's share of Clark County residents covered by commercial health insurance was the following - 1984/4%, 1985/18%, 1986/24%, 1987/17%, 1988/15%); *Barry v. Blue Cross of California*, 805 F.2d 866, 870-74 (9th Cir. 1986); *Ezpeleta v. Sisters of Mercy Hospital Corp.*, 800 F.2d 119, 121-122 (7th Cir. 1986).

(2) Humana Insurance contracted with Boulder City, Community, and St. Rose to be preferred providers in its insurance program;

(3) Donald Stewart, Executive Director of Sunrise, when testifying before the Nevada Senate, stated that the smaller primary care hospitals in Clark County do not compete against the large hospitals;[19]

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[19] More specifically, on May 20, 1987, Donald Stewart testified:

> Large, full service hospitals with the latest in medical technology compete vigorously against each other for market share. However, smaller primary care hospitals, charging less and not providing as specialized or technical care have found a niche in the market. For the most part, the larger hospitals do not compete against them.
>
> That will change with this bill. By forcing a rollback of our billed charges and a deep reduction in revenue per admission, the state will prevent us from being the kind of hospital we want to be, and force us and other large hospitals into markets we currently avoid.
>
> If you limit our prices and our revenue per admission, the only response left to us in the market is to increase our volume. This will require expanding into markets we don't currently pursue and competing directly for the segments of the market that are the mainstay of smaller hospitals. . . . The marketplace is dynamic and changing. You cannot change that by legislation. See Errata (#822), Exhibit 125, at 7-8.

(4) UMC did not compete in the same market since it charged lower prices for its services and, being a non-profit county hospital, was perceived to be the "indigent" hospital in Clark County;[20]

Errata (#822) at 290-305.

In analyzing Plaintiffs' second alleged market, the primary issue to be decided is whether Plaintiffs have presented sufficient evidence to support their allegation that the relevant service market for antitrust purposes consists of

[20] Plaintiffs also allege that physicians who practiced at Valley often did not admit its patients to UMC. See Errata (#822) at 299. Even if this fact is true, this Court fails to see how it helps to establish that UMC and Sunrise were not competitors.

Clark County's major for-profit acute care hospitals.[21] This Court finds that Plaintiffs have failed to do so.

Plaintiffs' assertion that Boulder City, Community, St. Rose, and Women's are not competitors of Sunrise because they do not offer a comparable range of hospital services misconstrues the alleged injury in this case. Although this Court would agree with the proposition that certain hospital services under certain facts may constitute a relevant submarket,[22] throughout this litigation Plaintiffs have not claimed or offered any evidence regarding the

[21] There is really no dispute as to the geographic market in this case. As established by Defendants' experts, the consumers in the market in which Sunrise competes regularly seek care at hospitals throughout Clark County. See Motion for Summary Judgement (#804) at 64-72; Reply (#841) Exhibit N, Supplemental Affidavit of Barry Harris, at 6-8, 13-14; Exhibit Z, Supplemental Affidavit of Harold M. Ting, at 21. Although Plaintiffs raise the point that UMC may be in a different geographic area than Sunrise, such an assertion is easily refuted by the analysis presented by Defendants' experts, the fact that UMC is only 3 miles away from Sunrise, and the fact that Valley Hospital, which Plaintiffs assert is a competitor of Sunrise, is in the same "western" location and is further from Sunrise than UMC. See *id.*; Reply (#841), Exhibit N, Supplemental Affidavit of Harold M. Ting, at Exhibit 4.

[22] *See, e.g., Morgan*, 924 F.2d at 1488-89 (radiology services constitute relevant market); *Oltz*, 861 F.2d at 1446-48 (patient anesthesiological services constitute relevant market.).

monopolization of specific hospital services such as open heart surgery or renal dialysis, etc. See Errata (#822) at 297. The antitrust injury alleged by Plaintiffs in this case is the actual or attempted monopolization of the general acute care hospital market in Clark County. See Errata (#822) at 313-315. Given that Boulder City, Community, St. Rose, and Women's all offer a variety of hospital services within this market, these hospitals have the ability to deprive Sunrise of significant levels of business and therefore, must be considered to be competitors of Sunrise. See Reply (#841) at 99-105; Errata (#822) at 296-97; *Morgan*, 924 F.2d at 1489 (competitors are "the group or groups of sellers or producers who have actual or potential ability to deprive each other of significant levels of business"); *Thurman*, 875 F.2d at 1374-1377 (specialty stores selling same products as more comprehensive home centers within same competitive market); *see also United States v. Rockford Memorial Corp.*, 898 F.2d 1278, 1284 (7th Cir. 1990), *cert. denied*, 498 U.S. 920, 111 S. Ct. 295, 112 L. Ed. 2d 249 (1990); *Weiss v. York Hosp.*, 745 F.2d 786, 826 (3rd. Cir. 1984), *cert. denied*, 470 U.S. 1060, 84 L. Ed. 2d 836, 105 S. Ct. 1777 (1985); *United States v. Carilion Health Service*, 707 F. Supp. 840, 847 (W.D. Va. 1989), *aff'd*, 892 F.2d 1042 (4th Cir. 1990).

Plaintiffs further allege that Boulder City, Community, and St. Rose are not competitors of Sunrise because these three hospitals are also preferred providers under the Humana insurance policy. Although it is true that to Humana Insurance insureds, having preferred provider status at Boulder City, Community, and St. Rose made these hospitals more competitive with Sunrise and with each other, the

simple fact remains that the acute care hospital service market in Clark County does not serve only Humana Insurance insureds. See *supra* at n. 18 (Humana Insurance's percentage of Clark County residents with medical insurance was 15% to 24% during relevant time period). Accordingly, because these institutions competed for patients independent of any relationship which they might have had with Humana Insurance, any argument raised by Plaintiffs that these hospitals did not compete in the same market as Sunrise must be rejected.[23]

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[23] Plaintiffs also rely on the affidavit of Victor Brewer and the testimony of Donald Stewart to support their argument that the smaller hospitals did not compete in the same market as Sunrise. With regard to Victor Brewer's affidavit, except for his statement regarding physician overlap between Community and Sunrise, the statements contained therein are conclusory allegations insufficient to raise a question of material fact. See Errata (#822), Exhibit 112. As to Donald Stewart's testimony, apart from being in the context of trying to sway the Nevada legislature from further regulating the hospital industry, the clear gravamen of these statements was that even though the larger hospitals in Clark County participated in the same marketplace as the smaller hospitals, they chose not to focus their marketing resources on pursuing this share of the market. See *supra* at n.19. Furthermore, to the extent that Stewart's statements could be inferred to support Plaintiffs' conclusion that Sunrise did not seek to compete vigorously against the smaller hospitals in Clark County, this statement alone does not raise a material question of fact with regard to whether these smaller hospitals acted as a competitive constraint on the same

Finally, Plaintiffs argue that UMC should not be considered a competitor of Sunrise because UMC charged lower prices than other hospitals, and because it is a non-profit county hospital, UMC was perceived to be the "indigent" hospital. This Court finds such arguments unpersuasive.

First, the facts in this case reveal that during the relevant time period UMC offered a wide range of acute care hospital services and had contracts for these services with nearly all managed care plans in Nevada. See Motion for Summary Judgment (#804) at 74-76; Exhibit #120; Exhibit #122; Exhibit #123; Exhibit #125 at 51-60. In addition, UMC competed with Sunrise for beds under Certificate of Need applications, attempted to recruit doctors from other Las Vegas hospitals, upgraded its facilities and equipment, and advertised to improve its public image. *Id.* at 77-78; Exhibit #122; Exhibit #125 at 60-62; Exhibit #129; Exhibit #131; Exhibit #133; see also Supplemental Affidavit of Harold M. Ting (#842), Exhibit 8 (UMC's 1989-1990 marketing plan which identifies Sunrise as a competitor). Such factors clearly demonstrate that UMC was a significant competitor for patients in Clark County's acute care hospital market.

With regard to Plaintiffs' pricing argument, as the Ninth Circuit has recognized before, "the scope of the relevant market is not governed by the presence of a price differential between competing products." *Twin City Sportserv., Inc. v.*

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general acute care hospital services offered by the larger hospitals.

Charles O. Finley & Co., 512 F.2d 1264, 1274 (9th Cir. 1975). This is particularly so where, as here, the pricing for hospital services commonly is subject to discounting through third-party payors, and where other factors such as location, physician affiliation, and the perceived quality of the institution are more likely to affect competition than price. See Motion for Summary Judgment (#804), Exhibit #8, Affidavit of Harold M. Ting at 9; Exhibit #120, at 17. Although it is no doubt true that being perceived as an "indigent hospital" negatively affected UMC's ability to attract commercially-insured patients, the facts also clearly demonstrate that this did not deter UMC from actively pursuing and providing services to them. See *supra* at 34-35; Reply (#841), Supplemental Affidavit of Barry C. Harris, Exhibit N, at 14-16; Supplemental Affidavit of Harold M. Ting, Exhibit Z, at 17-21; *see also Rockford Memorial Corp.*, 717 F. Supp. at 1284-85 (N.D. Ill. 1990); *Nonprofit Hospital Mergers: Proceed With Caution*, 20 Cumb. L. Rev. 719, 720-28 (1989-90) (describing generally the market forces which cause nonprofit hospitals to compete with for-profit hospitals). To argue that UMC, the second largest acute care hospital in Clark County, was entirely removed from the competitive market for hospital services because it was perceived by some as a "county hospital" which also served indigent patients simply ignores the competitive reality of the Clark County hospital market.

This Court concludes that, based on the record in this case, the Antitrust Class has failed to properly establish either of its proposed submarkets. To the extent a relevant competitive market is defined by the parties, this market would be the general acute care hospital service market in

Clark County consisting of Boulder City, Community, Desert Springs, St. Rose, Sunrise, UMC, Valley, and Women's hospitals.

5. Monopoly Power Within the Relevant Market

Once a relevant market is established, the inquiry shifts to whether the alleged monopolist exercised or had the potential to exercise market power. Although no single factor has been held determinative, the evaluation of market power usually depends on an evaluation of market share and barriers to entry. See *Oahu*, 838 F.2d at 366. A helpful framework for evaluating market share which has been used repeatedly by courts was articulated by Judge Learned Hand in *United States v. Alum. Co. of America*, 148 F.2d 416, 424 (2d Cir. 1945), where he found that a ninety percent market share is enough to constitute a monopoly, a sixty percent market share is doubtful, and "certainly thirty-three percent is not." *Id.*; *Syufy Enterprises v. American Multicinema, Inc.*, 793 F.2d 990, 995 (9th Cir. 1986), *cert. denied*, 479 U.S. 1031, 93 L. Ed. 2d 830, 107 S. Ct. 876 (1987); *Rebel Oil Co., v. Atlantic Richfield Co.*, 808 F. Supp. 1464, 1468 (D. Nev. 1992); *see also Barry*, 805 F.2d at 874; Areeda & Turner, *Antitrust Law*, § 518.3 at 522-523 (1991 Supp.). In addition, declining market share may reflect the absence of market power. Output as opposed to capacity is a more accurate measure of market share, and excess capacity, if it can be reasonably be determined, may limit the exercise of market power. Oahu, 838 F.2d at 366; Carilion, 707 F. Supp. at 848-49; Areeda & Turner, Antitrust Law, §§ 520(a), 521(e) (1991 Supp.).

In determining the market share of hospitals, facts that need to be examined include the percentage shares of patient days and admissions of the various competitors. Facts which are relevant to hospital capacity include the number of licensed beds within the hospitals as well as their respective occupancy percentages. An examination of these facts in the present case reveal the following:

(1) Patient Days - Sunrise's market share of Clark County hospital patients in terms of patient days was 35% in 1985, 33% in 1986, 32% in 1987, and 32% 37 in 1988;[24]

(2) Admissions - Sunrise's market share of Clark County hospital patients in terms of admissions was 34% in 1985, 32% in 1986, 30% in 1987, and 30% in 1988;[25]

(3) Number of Licensed Beds - Sunrise's percentage of licensed beds in Clark County was 36% in 1985, 35% in 1986, 35% in 1987, and 35% in 1988;[26]

(4) Occupancy Percentage - Sunrise's capacity utilization of all beds was 52% in 1985, 48% in 1986, 52% in 1987, and 53% in 1988 compared to a Clark County average utilization

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[24] See Motion for Summary Judgement (#804), Exhibit 7, Affidavit of Barry C. Harris at Exhibit 2a; Reply (#841), Supplemental Affidavit of Barry C. Harris at Exhibit 1.

[25] *Id.*

[26] Reply (#841), Supplemental Affidavit of Barry C. Harris at Exhibit 1.

rate of 53% in 1985, 44% in 1986, 56% in 1987, and 58% in 1988.[27]

As can be seen from the above figures, throughout the relevant time period, Sunrise had approximately one third of the Clark County market for acute care hospital services. Although it is true that at the time in question, Nevada's Certificate of Need requirements acted as a potential entry barrier to new participants, the evidence also demonstrates that during the relevant time period there was substantial excess bed capacity at existing hospitals. This excess capacity would temper any competitive advantage Sunrise might have realized from such regulations.[28]

Accordingly, this Court further finds that Plaintiffs have failed to establish that Defendants had the ability to exercise monopoly power within Clark County's acute care hospital market during the relevant time period. *See Thurman*, 875 F.2d, at 1377-80; *Barry*, 805 F.2d at 874; *Bhan*, 669 F.

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[27] See Motion for Summary Judgment (#804), Exhibit 7, Affidavit of Barry C. Harris at Exhibit 4. Also included in his exhibit are the individual utilization rates of the various individual hospitals in Clark County. These rates are approximately 40% to 60% during the relevant time period.

[28] In fact, an argument could be made that the Nevada's Certificate of Need requirements actually injured Sunrise's competitive position since of the 2415 additional beds which were licensed between 1979 and 1988, only 16 of these were given to Sunrise. See Reply (#242), Supplemental Affidavit of Harold M. Ting, Exhibit Z, at Exhibit 5.

Supp. at 1022-23.[29] Using only Desert Springs, Sunrise, Valley, and UMC as the competitive market, in total patient days Humana had market shares of 40% in 1985, 38% in 1986, 36% in 1987, and 36% in 1988 and in admissions Humana had market shares of 38% in 1985, 39% in 1986, 36% in 1987, and 35% in 1988. See Opposition (#820), Exhibit 107 (data relating to inpatient days and admissions for Desert Springs, Sunrise, Valley, and UMC).

6. Antitrust Injury

Finally, even if this Court could find that Plaintiffs have adequately defined a relevant competitive market and have shown Defendants monopolized or attempted to monopolize that market, this Court would nevertheless have to conclude that Plaintiffs have not adequately demonstrated antitrust injury. Although Plaintiffs claim that they have suffered antitrust injury as a result of paying excessive premiums and/or co-payments, such monetary losses relate, if at all, to Defendant Humana Insurance's allegedly erroneous interpretation of the health insurance contract, and not the alleged anti-competitive acts of Sunrise (e.g. the diversion of critical care patients and refusal to unbundle level III neonatal services). This, coupled with the fact that Plaintiffs were only indirect purchasers of hospital services whose

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[29] Even if this Court were to assume that the smaller hospitals in Clark County were not within the same competitive market as Sunrise, such a fact does not alter the conclusion that Sunrise did not have sufficient market share within Clark County to exercise market power during the relevant time period.

premiums and co-payments were undoubtedly subject to a number of other influences (e.g. competition in the health insurance market, state regulation, etc.) external to the cost of hospital services, clearly demonstrates that Plaintiffs' Antitrust Class has not suffered "antitrust injury." *Cf. Austin*, 903 F.2d at 1390-93.

C. Conclusion

In light of Plaintiffs' failure to adequately demonstrate the alleged submarkets, the fact that Defendants could not exercise market power, and the failure of Plaintiffs to demonstrate antitrust injury, Defendants are entitled to summary judgment on Plaintiffs' Second Claim for Relief.

IV. RICO

A. Overview

The Third and final Claim for Relief alleged by Plaintiffs in their Second Amended Complaint is a violation of RICO pursuant to 18 U.S.C. § 1964(c) brought by Plaintiffs Co-Payor Class and Premium Payor Class. In support of this claim, both classes of Plaintiffs assert that during 1985 to 1988: (1) Humana and Humana Insurance acted as members of an "association in fact" enterprise or, alternatively, that Humana, a person within the meaning of 18 U.S.C. § 1964(3), associated with Humana Insurance, an enterprise within the meaning of 18 U.S.C. § 1961(4); (2) that Humana and Humana insurance entered into a secret agreement to discount amounts owed by Humana Insurance for hospital charges incurred by Humana Insurance insureds; (3) Humana and Humana Insurance concealed and/or misrepresented this agreement to Plaintiffs in the numerous

mailings, telephone calls, and television/radio advertisements; and (4) that such acts (a) were intended to and did induce the Premium Payor Class into buying policies they would not have otherwise purchased, and (b) defrauded the Co-Payor Class into paying excessive co-payments. Second Amended Complaint (#370) at 18, 22-23; Errata (#822) at 103-109, 221-223, 251-253.

In support of their Motion for Summary Judgment (#804), Defendants Humana and Humana Insurance assert the following arguments:

(1) Plaintiffs cannot prove that the alleged misrepresentations and/or omissions of Defendants were intended to defraud them since (a) it is undisputed that the Humana Insurance policy mirrored other health plans in place during the relevant time period and, (b) the discount arrangement was disclosed to Nevada regulators. Motion for Summary Judgment (#804) at 98-116;

(2) New precedent requires this Court to reverse its prior ruling that Humana is a "person" distinct from Humana Insurance, the "enterprise," under 18 U.S.C. § 1962(c). Motion for Summary Judgment (#804) at 131-136;

(3) The Premium Payor Class cannot show that they suffered RICO injuries, or even assuming they could, that these injuries were proximately caused by Defendants' actions. Motion for Summary Judgment (#804) at 116-126; (4) Under RICO, the only damages which the Co-Payor Class is entitled to recover is the amount of excess co-payments which the class paid as a result of Defendants' allegedly improper interpretation of the policy. Motion for Summary

Judgment (#804) at 126-131; (5) The McCarran-Ferguson Act Precludes Plaintiffs' RICO cause of action. Motion for Summary Judgment (#804) at 136-146.

B. Requirements of RICO, 18 U.S.C. § 1962(c)

Liability pursuant to 18 U.S.C. § 1962(c) requires a Plaintiff to demonstrate (1) the conduct (2) of an enterprise (3) through a pattern (4) of racketeering activity. *Sun Savings and Loan Assoc. v. Dierdorff*, 825 F.2d 187, 191 (9th Cir. 1987). "Racketeering activity" is any act indictable under various provisions of Title 18 of the United States Code, see 18 U.S.C. § 1961, and includes the predicate acts alleged in this case of mail fraud and wire fraud pursuant to 18 U.S.C. §§ 1341, 1343. *Id.*

In order to commit mail fraud under § 1341, Plaintiffs must demonstrate that: "(1) the defendants formed a scheme or artifice to defraud: (2) the defendants used the United States mails or caused a use of the United States mails in furtherance of the scheme; and (3) the defendants did so with the specific intent to deceive or defraud." *Schreiber Distributing v. Serv-Well Furniture Co.*, 806 F.2d 1393, 1400 (9th Cir. 1986). Similarly, in order to commit wire fraud pursuant to § 1343, Plaintiffs must show: "(1) the formation of a scheme or artifice to defraud; (2) the use of the United States wires or causing a use of the United States wires in furtherance of the scheme; and (3) specific intent to deceive or defraud." *Id.*

Under these provisions, fraud refers to "'(1) a false representation (2) in reference to a material fact (3) made with knowledge of its falsity (4) and with the intent to

deceive.'" *United States v. Bonallo*, 858 F.2d 1427, 1433 (9th Cir. 1988). A failure to disclose or concealment may also serve a basis for a fraudulent scheme under mail and wire fraud statutes; however, such nondisclosure must arise from a failure to comply with an independent statutory or fiduciary duty. *California Arch. Bldg. Prods. v. Franciscan Ceramics*, 818 F.2d 1466, 1472 (9th Cir. 1987), *cert. denied*, 484 U.S. 1006, 98 L. Ed. 2d 650, 108 S. Ct. 698 (1988); *United States v. Dowling*, 739 F.2d 1445, 1449 (9th Cir. 1984), *rev'd on other grounds*, 473 U.S. 207, 87 L. Ed. 2d 152, 105 S. Ct. 3127 (1985).

C. Plaintiffs' Failure to Demonstrate Intent to Defraud

The first argument raised by Defendants in support of their Motion for Summary Judgment (#804) on Plaintiffs' RICO claims is that there is no evidence to suggest that Defendants had "specific intent to deceive or defraud" the Plaintiffs. More specifically, Defendants contend that insofar as the development of the policy in question is concerned:

(1) during the period in question, the health care industry was just beginning to experiment with preferred provider arrangements, and no industry standards were in place governing how co-payment discounts should be administered; (2) that the policy in question was simply modeled after one developed and already in place in Florida; (3) Medicare employs an analogous system for the payment of co-payments; (4) confidentiality clauses were common in the industry for such discount arrangements to ensure the protection of proprietary information; and (5) that only 9.54% of all Humana Insurance insureds resulted in paying

higher co-payments if the discount given Humana Insurance would have been shared.

Motion for Summary Judgment (#804) at 99-108.

Defendants further assert that various parties other than Defendants were aware of the co-payment arrangement, that Defendants disclosed the discount arrangement to Nevada regulators at various times between 1984 and 1986, and that on June 6, 1986, while at hearings before the Nevada Department of Insurance ("DOI"), Defendants directly informed DOI officials that bills issued to insureds did not reflect any discounts received by Humana Insurance from Humana. Motion for Summary Judgment (#804) at 110-116.

Plaintiffs respond that notwithstanding any disclosures made by Defendants to third parties, Defendants failed to disclose the co-payment scheme to Plaintiffs directly, and even had attempted to conceal it. According to Plaintiffs, Defendants' Motion for Summary Judgment (#804) raises a material question of fact regarding whether Defendants had the requisite "intent" required under the mail and wire fraud statutes. See Errata (#822) at 146, 219-221.

In determining whether Defendants satisfy the "intent" requirement of the mail and wire fraud statutes, Plaintiffs must demonstrate that the fraud was active and not merely constructive. *United States v. Bohonus*, 628 F.2d 1167, 1172 (9th Cir. 1980), *cert. denied*, 447 U.S. 928, 65 L. Ed. 2d 1122, 100 S. Ct. 3026 (1980). Specific intent may be shown by examining the scheme to determine whether it "was reasonably calculated to deceive persons of ordinary prudence and comprehension." *Sun Savings*, 825 F.2d at

196; *Schreiber*, 806 F.2d at 1400; *Bohonus*, 628 F.2d at 1172; *see also United States v. Green*, 745 F.2d 1205, 1209 (9th Cir. 1984), *cert. denied*, 474 U.S. 925, 88 L. Ed. 2d 266, 106 S. Ct. 259 (1985) (specific intent to defraud means to deceive or mislead, and does not include situations where act involved was the result of ignorance, mistake, or accident).

Notwithstanding the facts asserted by Defendants, viewing all inferences in favor of Plaintiffs as this Court must do, it is clear that a material question of fact exists as to whether Defendants' interpretation of the co-payment provision, which was furthered through the use of United States mails or wires, was specifically intended to mislead or deceive Plaintiffs or whether it was the result of ignorance, mistake, or accident. Accordingly, Defendants Motion for Summary Judgment (#804) on this ground must be denied.

D. Person/Enterprise Distinction

Plaintiffs next allege that Humana and Humana Insurance acted as members of an "association in fact" enterprise or, alternatively, that Humana, a person within the meaning of 18 U.S.C. § 1964(3), associated with Humana Insurance, an enterprise within the meaning of 18 U.S.C. § 1961(4), to defraud Plaintiffs. As this Court recognized in its July 9, 1991 Order (#364), such allegations satisfy the "person/enterprise" requirements of U.S.C. § 1962(c). *See River City Markets v. Fleming Foods West*, 960 F.2d 1458, 1460-62 (9th Cir. 1992); *U.S. v. Feldman*, 853 F.2d 648, 656-59 (9th Cir. 1988), *cert. denied*, 489 U.S. 1030, 103 L. Ed. 2d 222, 109 S. Ct. 1164 (1989); *cf. Glessner v. Kenny*, 952 F.2d 702, 710-14 (3rd Cir. 1991). Accordingly,

Defendants are not entitled to summary judgment on this ground.

E. RICO Damages for the Premium Payor Class

The third argument asserted by Defendants in support of summary judgment is that the Premium Payor Class has not demonstrated the requisite causal nexus between the RICO damages they seek and the fraud alleged as a predicate act. This Court agrees.

At the outset, the Premium Payor Class notes that the nondisclosure of the discounting arrangement between Humana and Humana Insurance induced its members into obtaining Humana Insurance policies. Errata (#822) at 221-222. The Premium Payor Class then asserts that numerous television and direct mail advertisements promised that employers and employees would "share in reduced hospital costs through premium reductions." Errata (#822) at 104, 222, 235.[30] The damages which the Premium Payor

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[30] The language from the advertisements to which the Premium Payor Class refer is as follows:

(1) "Tap Dance" commercial: "Because we can control costs, both you and your company can save a great deal of money without the song and dance;"
(2) "Mumbo Jumbo" commercial: "We can save you and your company money without insurance mumbo jumbo;"
(3) "Cuts" commercial: "Humana is a hospital company, so we can control costs,
saving employers and employees money."

Class contend they have suffered as a result of these actions is "the difference in the actual price of the insurance product that they purchased (the premiums actually paid) and what the premiums should have been if premiums were calculated to include the discounts that Humana Insurance was receiving." Errata (#822) at 222.

In *Imagineering, Inc. v. Kiewit Pacific Co.*, 976 F.2d 1303 (9th Cir. 1992), *cert. denied*, 123 L. Ed. 2d 266, 113 S. Ct. 1644 (1993), the Ninth Circuit clarified the requirements of proximate causation and recoverable damages under RICO. With respect to the issue of proximate causation, the Court in Imagineering stated:

> In order to maintain a cause under RICO then, the plaintiff must show not only that the defendant's violation was a "but for" cause of his injury, but that it was the proximate cause as well. This requires that there must be a direct relationship between the injury asserted and the injurious conduct alleged. . . . One principle underlying this requirement is that the less direct the injury is, the more difficult it becomes to ascertain the amount of a plaintiff's damages attributable to the violation, as distinct from other, independent factors.

Id. at 1311, citing *Holmes v. Securities Investor Protection Corp.*, 117 L. Ed. 2d 532, 112 S. Ct. 1311, 1317-18 (1992). Consistent with this view of proximate cause, the Court in

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Errata (#822) at 33; Reply (#841) at 35.

Imagineering held that not all injuries are compensable under RICO, but rather only those which are proven by a "concrete financial loss." *Id.* at 1310; *see also Oscar v. University Students Co-operative Ass'n*, 965 F.2d 783, 785 (9th Cir. 1992) (*en banc*), *cert. denied*, 121 L. Ed. 2d 581, 113 S. Ct. 655 (1992).

The claim by the Premium Payor Class that Humana Insurance's advertising campaign misrepresented that its members would "share in reduced hospital costs through premium reductions," is without merit. The advertisements to which Plaintiffs refer are nothing more than puffery and plainly do not contain any representations which would indicate how insurance premiums would be calculated. As such, this alleged predicate act cannot be viewed as the proximate cause of the Premium Payor Class' asserted RICO injury. *See, e.g., Berent v. Kemper Corp.*, 973 F.2d 1291, 1295-96 (6th Cir. 1992).

The Premium Payor Class also argues that had its members known how Humana Insurance would calculate co-payments under its policy, they would not have purchased Humana polices. This argument is problematic for several reasons.

First, as stated by Defendants, the logical injury which would follow from a "fraudulent inducement" would be that the Premium Payor Class could have obtained comparable policies elsewhere for less and instead chose Humana as their insurance carrier. See Reply (#841) at 36-37. Plaintiffs do not present any evidence in this regard. See Errata (#822) at 227-236.

Plaintiffs argue instead that they did not get the benefit of what they bargained for: an insurance policy under which co-payments would be calculated with consideration of any hospital discounts received. See Errata (#822) at 224. Plaintiffs may be correct, although there is no evidence in the record to demonstrate that they are. Unlike the Co-Payor Class, however, the members of the Premium Payor Class never incurred a hospital bill or made a co-payment. Its members simply paid the premiums they had previously agreed to pay assuming that any hospital discounts obtained by Humana Insurance would be reflected in their co-payment determinations. Accordingly, even though it is clear that the members of the Premium Payor class could have *potentially* been defrauded by Defendants' calculation of co-payments, under the undisputed facts, no such injury actually occurred.

Plaintiffs' additional argument that their "injury" arose from the fact that Sunrise's discount to Humana Insurance should have been included in calculating their initial premiums must be rejected for two reasons. First, although this argument enables Plaintiffs to state a more direct injury, it completely ignores the fact that the fraud which supposedly induced Plaintiffs to purchase the Humana policy involved representations about how co-payments were to be determined. Nothing in the record suggests that the Defendants fraudulently represented how they would calculate premiums.[31] Even if Humana Insurance did err in

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[31] Indeed, Plaintiffs indirectly acknowledge this point in their Opposition when they admit that their argument is not that "Defendants' fraud 'caused' Defendants to charge improper premiums." See Errata (#822) at 224.

its calculation of the initial premiums, this error was independent of the predicate acts which Plaintiffs have alleged throughout this litigation.

Second, Plaintiffs' argument ignores other factors, such as the price of competing insurance products, that were used to determine the price of premiums paid by the Premium Payor Class. See Errata (#822) at 228. Assuming that simply deducting the cost of the discounts in Defendants' actuarial formula would lead to a fair market price that the Premium Payor Class would have paid for the Humana policy ignores the competitive realities of the group health insurance market.

Accordingly, given the failure of the Premium Payor Class to adequately demonstrate a "direct relationship between the injury asserted and the injurious conduct alleged" or a "concrete financial injury" for purposes of RICO, Defendants are entitled to summary judgment. *Imagineering*, 976 F.2d at 1311.

F. RICO Damages for the Co-Payor Class

This Court previously has analyzed how to calculate the damages incurred by the Co-Payor Class which would be causally connected to Defendants' alleged breach of fiduciary duties under ERISA. *See supra* at 1507-09. This Court finds that the reasoning of the Ninth Circuit in *Imagineering* and *Oscar* regarding the requirements of proximate cause and concrete financial injury applies with equal force in the context of damages available under RICO. Accordingly, Defendants are entitled to summary judgment on the Co-Payor Class' RICO claim to the extent that the Co-Payor

Class seeks damages in excess of the amount that they could have recovered under a proper interpretation of the co-payment provision.

G. McCarran-Ferguson Act Exception

Finally, Defendants argue that the McCarran-Ferguson Act precludes Plaintiffs from pursuing their RICO claim. This Court agrees.

Section § 1012(b) of the McCarran-Ferguson Act provides:

> No Act of Congress shall be construed to invalidate, impair or supersede any law enacted by any State for the purpose of regulating the business of insurance, . . . unless such Act specifically relates to the business of insurance. . . .

15 U.S.C. § 1012(b).

The purpose of this provision is to impose a clear-statement rule "that state laws enacted 'for the purpose of regulating the business' do not yield to conflicting federal statutes unless a federal statute specifically requires otherwise." *United States Dept. of the Treasury v. Fabe*, 124 L. Ed. 2d 449, 113 S. Ct. 2202, 61 U.S.L.W. 4579, 4583 (1993) (citation omitted).

In order to determine if Section 2(b) precludes application of a particular federal statute, a court must apply the following four part test: (1) is the federal statute one which specifically relates to the business of insurance; (2) does the complained of activity constitute the business of insurance; (3) has the state enacted laws for the purpose of

regulating the complained of activity; and (4) would the application of the federal statute invalidate, impair, or supersede the state legislation. *See LeDuc v. Kentucky Central Life Ins. Co.*, 814 F. Supp. 820, 826-27 (N.D. Cal. 1992); *Thacker v. New York Life Ins. Co.*, 796 F. Supp. 1338, 1341 (E.D. Cal. 1992).

There can be no dispute that RICO does not specifically relate to the business of insurance. *See LeDuc*, 814 F. Supp. at 827; *Thacker*, 796 F. Supp. at 1341.

It is equally clear that Plaintiffs' complained of activities constitute the "business of insurance." The crux of the RICO claim brought by the Premium Payor Class is that nondisclosure of the discount arrangement between Humana and Humana Insurance in advertisements and other communications fraudulently induced its members into purchasing the Humana Insurance policies. See Second Amended Complaint (#370) at 18-23. The gravamen of the RICO claim brought by the Co-Payor Class is that nondisclosure of the discount arrangement between Humana and Humana Insurance fraudulently induced its members into paying excessive co-payments. *Id.* Since these alleged acts directly relate to the purchase and execution of Humana Insurance policies, they fall squarely within the meaning of the "business of insurance." *See Fabe*, 61 U.S.L.W. at 4582 ("the actual performance of an insurance contract falls within 'the business of insurance'. . . to hold otherwise would be mere formalism."); *WEXCO, Inc. v. IMC, Inc.*, 820 F. Supp. 194 (M.D. Pa. 1993); *LeDuc*, 814 F. Supp. at 828; *but see, Thacker*, 796 F. Supp. at 1342. Accordingly,

Defendants also satisfy the second prong of the McCarran-Ferguson test.

In evaluating whether Nevada has enacted laws governing the activities complained of in Plaintiffs' RICO claims, it is not necessary that a state have a specific statute banning a particular practice. The inquiry is whether the state involved has adopted a regulatory scheme which possesses jurisdiction over the challenged activity. *See Feinstein v. Nettleship Co. of Los Angeles*, 714 F.2d 928, 933 (9th Cir. 1983), *cert denied*, 466 U.S. 972, 80 L. Ed. 2d 820, 104 S. Ct. 2346 (1984); *WEXCO*, 820 F. Supp. at 197.

In 1971, Nevada adopted the Trade Practices and Frauds; Financing of Premiums Act ("Act"), N.R.S. § 686A.010, *et seq.* This Act was designed to regulate the business of insurance as recognized in the McCarran-Ferguson Act "by defining, or providing for the determination of, all such practices in this state which constitute unfair methods of competition or unfair or deceptive acts or practices and by prohibiting the trade practices so defined and determined." N.R.S. § 686A.010. Under this Act, Nevada recognized that the Commissioner of Insurance "has exclusive jurisdiction" in regulating the trade practices of the business of insurance. N.R.S. § 686A.015. Furthermore, the Act specifically prohibits the dissemination of false or misleading information relating to the sale of policies or the benefits obtained thereto, N.R.S. §§ 686A.030, 686A.040, and those violating the Act are subject to enforcement of its provisions by the Commissioner. N.R.S. §§ 680A.200, 686A.160, 686A.170,

and 686A.310. These statutes and others requiring the approval of forms used by insurers, N.R.S. §§ 687B.120, 687B.130, clearly demonstrate an attempt by Nevada to regulate the conduct complained of in this case through the adoption of a comprehensive regulatory scheme.[32] Accordingly, this Court finds that the third prong of McCarran-Ferguson test is also satisfied. *See also WEXCO*, 820 F. Supp at 197; *LeDuc*, 814 F. Supp. at 829; *Thacker*, 796 F. Supp at 1341-42.

Finally, under the McCarran-Ferguson test, this Court must consider whether the application of RICO would invalidate, impair, or otherwise supersede Nevada legislation. As acknowledged by Plaintiffs in their Opposition (#822), the Nevada Commissioner of Insurance, in enforcing the Act, can prohibit unfair trades practices, impose fines, and withdraw approval of insurance plans. See Opposition (#822) at 270-274; N.R.S. §§ 686A.160, 686A.170, 686A.187, and 687B.130. As Plaintiffs contend, the State of Nevada does not proscribe the same harsh penalties which would be available under RICO (e.g. treble damages and attorney's fees) for Defendants' violation of unfair trade practices. See Errata (#822) at 270-274. As recognized by the courts in *WEXCO* and *LeDuc*, however, this is the very

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[32] In fact, Plaintiffs tacitly admit that Humana Insurance's fraudulent activity was covered by the existing Nevada statutory scheme when they argue that Humana Insurance "cavalierly ignored" these "general legal requirements," thus requiring the Nevada legislature to pass in 1987 the more rigorous requirements of sharing discounts under AB 289. See Opposition (#822) at 271.

reason why the fourth prong of the McCarran-Ferguson test is met. *See WEXCO* 820 F. Supp. at 198; *LeDuc*, 814 F. Supp at 829; *but see Thacker*, 796 F. Supp at 1343. For this Court to allow Plaintiffs to pursue the harsher RICO penalties for behavior which the Nevada Legislature apparently did not deem worthy of such penalties, would be tantamount to allowing Congress to intercede in an area expressly left to the states under the McCarran-Ferguson Act. Accordingly, this Court also finds that the fourth prong of the McCarran-Ferguson analysis is satisfied.[33]

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[33] At oral argument, Plaintiffs for the first time argued that since Defendants' conduct could arguably be sanctioned under Nevada's RICO statute, N.R.S. @ 207.470, the harsh remedies afforded by the federal RICO statute cannot be seen to displace or supersede existing Nevada law. This Court disagrees.

As recognized earlier in this Order, Plaintiffs could not pursue a Nevada RICO claim in this case given the fact that ERISA preempts such a claim. *See supra* at __. Beyond this, it is clear that in adopting the Act, the Nevada legislature intended the Insurance Commissioner to have "exclusive jurisdiction" over fraudulent insurance practices. See N.R.S. @ 686A.015. This, combined with the fact that the Nevada legislature specifically identified fraud on the part of the insured and not by the insurer as a possible predicate act under N.R.S. @ 207.470, demonstrates an intent of the Nevada legislature to limit the amount of damages recoverable against an insurer for fraud. *See* N.R.S. @@ 207.360(30), 680A.200, 686A.160, 686A.170, 686A.290, 686A.291, and 686A.310.

This Court, therefore, concludes that the RICO claims alleged by the Premium Payor Class and the Co-Payor Class are barred by the McCarran-Ferguson Act. In doing so, the Court recognizes that Plaintiffs may view the interplay between the McCarran-Ferguson Act and ERISA as posing a preemptive "Catch-22," with McCarran-Ferguson precluding Plaintiffs' federal RICO claim and ERISA barring Plaintiffs' state RICO claim. The result is not entirely anomalous, however, given that Congress has occupied the field through ERISA by which certain well-defined remedies have been provided and others excluded.

V. ORDERS

IT IS THEREFORE ORDERED that Defendants' Motion for Summary Judgment (#804) is Granted to the extent that Judgment is hereby entered in favor of Defendants and against Plaintiffs as to Plaintiffs' First, Second, and Third Claims for Relief.

IT IS FURTHER ORDERED that Plaintiffs' Co-Payor Class only shall have to and including August 24, 1993, within which to file a Third Amended Complaint seeking relief on Plaintiffs' First Claim for Relief pursuant to 29 U.S.C. § 1132(a)(1)(B), consistent with Part II of this Memorandum Decision.

DATED: July 21, 1993

PHILIP M. PRO
United States District Judge

APPENDIX C
(Relevant Statutory Provisions)

McCarran-Ferguson Act, 15 U.S.C. § 1012(b):

> No Act of Congress shall be construed to invalidate, impair, or supersede any law enacted by any State for the purpose of regulating the business of insurance, or which imposes a fee or tax upon such business, unless such Act specifically relates to the business of insurance: *Provided,* That after June 30, 1948, the Act of July 2, 1890, as amended, known as the Sherman Act, and the Act of October 15, 1914, as amended, known as the Clayton Act, and the Act of September 26, 1914, known as the Federal Trade Commission Act, as amended [15 U.S.C. 41 et seq.], shall be applicable to the business of insurance to the extent that such business is not regulated by State Law.

Racketeer Influenced and Corrupt Organizations Act, 18 U.S.C. § 1962:

> (a) It shall be unlawful for any person who has received any income derived, directly or indirectly, from a pattern of racketeering activity or through collection of an unlawful debt in which such person has participated as a principal within the meaning of section 2, title 18, United States Code, to use or invest, directly or indirectly, any part of such income, or the proceeds of such income, in acquisition of any interest in, or the establishment or operation of, any enterprise which is engaged in,

or the activities of which affect, interstate or foreign commerce. A purchase of securities on the open market for the purposes of investment, and without the intention of controlling or participating in the control of the issuer, or of assisting another to do so, shall not be unlawful under this subsection if the securities of the issuer held by the purchaser, the members of his immediate family, and his or their accomplices in any pattern or racketeering activity or the collection of an unlawful debt after such purchase do not amount in the aggregate to one percent of the outstanding securities of any one class, and do not confer, either in law or in fact, the power to elect one or more directors of the issuer.

(b) It shall be unlawful for any person through a pattern of racketeering activity or through collection of an unlawful debt to acquire or maintain, directly or indirectly, any interest in or control of any enterprise engaged in , or the activities which affect, interstate or foreign commerce.

(c) It shall be unlawful for any person employed by or associated with any enterprise engaged in, or the activities which affect, interstate or foreign commerce, to conduct or participate, directly or indirectly, in the conduct of such enterprise's affairs through a pattern of racketeering activity or collection of unlawful debt.

(d) It shall be unlawful for any person to conspire to violate any of the provisions of subsection (a), (b), (c) of this section.

Racketeer Influenced and Corrupt Organizations Act, 18 U.S.C. § 1964:

(a) The district courts of the United States shall have jurisdiction to prevent and restrain violations of section 1962 of this chapter by issuing appropriate orders, including, but not limited to: ordering any person to divest himself of any interest, direct or indirect, in any enterprise; imposing reasonable restrictions on the future activities or investments of any person, including but not limited to, prohibiting any person from engaging in the same type of endeavor as the enterprise engaged in, the activities of which affect interstate or foreign commerce; or ordering dissolution or reorganization of any enterprise, making due provision for the rights of innocent persons.

(b) The Attorney General may institute proceedings under this section. Pending final determination thereof, the court may at any time enter such restraining orders or prohibitions, or take such other actions, including the acceptance of satisfactory performance bonds, as it shall deem proper.

(c) Any person injured in his business or property by reason of a violation of section 1962 of this

chapter may sue therefor in any appropriate United States district court and shall recover threefold the damages he sustains and the cost of the suit, including a reasonable attorney's fee.

(d) A final judgment or decree rendered in favor of the United States in any criminal proceeding brought by the United states under this chapter shall estop the defendant from denying the essential allegations of the criminal offense in any subsequent civil proceeding brought by the United States.